NCE EXAM PREP

Practice Exam 02

170 Practice Questions

225 minutes | A, B, C answer choices

Questions followed by answer key and detailed explanations

> **Instructions.** Select the single best answer for every question. Record your answers separately. Work at exam pace, allowing approximately 225 minutes for all 170 questions. The answer key and explanations begin in Part B; avoid viewing them until you have completed Part A.

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PART A — QUESTIONS

Choose one answer (A, B, or C) for each question.

Question 1

> An integrated-care counselor considers pasting a client's safety plan into a free AI translation website. The site retains submitted text, warns that translations may be inaccurate, and is not approved by the organization. Policy requires qualified language assistance for clinically significant communication.

What should the counselor do?

A) Use a qualified language service and keep the safety plan out of the unapproved tool

B) Remove the client's name and use the website because de-identification guarantees an accurate translation

C) Use the website first and ask a family member to correct any mistakes later

Question 2

> A client brings a close friend to intake for support. The friend begins answering every question and asks to receive appointment reminders and copies of future assessment results. The client has not yet stated what involvement is wanted.

What should the counselor do?

A) Accept the friend's requests because bringing a support person grants access to future information

B) Exclude the friend immediately without asking the client about preferences

C) Speak directly with the client about the friend's desired role, privacy boundaries, and any specific permission before involving the friend

Question 3

> An adult arrives at a crisis center after a sexual assault and becomes visibly overwhelmed as several available services are described. The person is medically stable and says, “Please stop deciding things for me.”

What should the counselor do?

A) Complete the most time-sensitive referrals first and explain the decisions afterward.

B) Pause, acknowledge the loss of control, and present one option at a time while requesting consent before each next step.

C) Ask the person to choose immediately from the full list so no service opportunity is missed.

Question 4

> Bao receives prescription instructions written in English but prefers health information in Vietnamese. He tells his counselor that he is unsure whether to take the medication with food. He has no acute symptoms, and an authorization permits communication with the pharmacy and prescriber.

What should the counselor do?

A) Document Bao's uncertainty and coordinate qualified Vietnamese language support with the pharmacist or prescriber for the medication instructions

B) Enter the instructions into a general translation application and explain the resulting directions to Bao

C) Infer the dosing instruction from the picture on the label and tell Bao what it most likely means

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Question 5

> A client who pulls hair when studying notices an urge only after reaching toward the scalp. The client wants a concrete behavioral skill and agrees to track the pattern.

Which intervention is the best fit?

A) Ask the client to analyze what the hair pulling symbolizes before changing the behavior.

B) Practice early urge awareness and a competing response that makes pulling difficult for a brief period.

C) Recommend studying only in public settings where embarrassment will prevent the behavior.

Question 6

Which process is most useful for creating an individualized professional development plan?

A) Select the same annual courses as experienced colleagues regardless of current practice needs

B) Choose training primarily in areas where the counselor already feels confident and successful

C) Integrate self-assessment, client-population needs, outcome information, and feedback from qualified colleagues

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Question 7

> During office hours, a caller makes a credible bomb threat against a counseling center. The center's written emergency plan requires immediate evacuation, a call to emergency services, and assembly at a designated location.

What should the clinical director do?

A) Finish sessions before evacuating so abrupt endings do not distress clients

B) Search the building personally before deciding whether the threat is credible

C) Activate the emergency plan, evacuate, contact responders, and account for clients and staff

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Question 8

An electronic health record audit shows that a scheduling employee opened a neighbor's counseling chart despite having no work-related reason. The clinic's security policy requires immediate access control, preservation of audit evidence, and privacy-officer review.

What is the most appropriate initial response?

A) Restrict the employee's access, preserve the audit log, and report the event through the required review process

B) Ask the employee to promise not to repeat the search and remove the event from the audit log

C) Leave access unchanged until the neighbor confirms that the employee used the information outside the clinic

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Question 9

A client has a history of a three-week major depressive episode. The client also reports a distinct five-day period of elevated mood, decreased need for sleep, increased talkativeness, and unusual goal-directed activity that was observable to others but caused no marked impairment, psychosis, or hospitalization. There has never been a manic episode, and substances and medical causes have been ruled out.

Which diagnosis is most consistent with this history?

A) Bipolar I disorder

B) Bipolar II disorder

C) Persistent depressive disorder

Question 10

A counselor enters credentials after clicking a convincing password-reset link and then notices the web address was not the clinic's domain. Security policy requires immediate incident reporting, password reset through a verified channel, session revocation, and device review.

What should the counselor do?

A) Follow the incident procedure immediately rather than waiting for evidence that the account was misused

B) Delete the message and take no further action if the account still opens normally

C) Send the suspicious link to coworkers so they can test whether it also accepts their credentials

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Question 11

> A first-generation college student says family members view counseling as selfish because personal problems should remain within the family. The student values family loyalty and also wants help managing stress.

Which intervention is most culturally responsive?

A) Explore both commitments and help the student frame coping goals in a way that can support personal well-being and valued family relationships.

B) Encourage the student to keep counseling secret so family attitudes do not interfere with treatment.

C) Challenge family loyalty as a barrier to the independence needed for emotional health.

Question 12

Which content in a transfer summary most directly supports continuity with the next counselor?

A) A chronological copy of every progress note, including material unrelated to current care

B) Current goals, interventions and responses, important client preferences, and pending follow-up needs

C) The diagnosis and appointment count without information about the client's current plan

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Question 13

> Clients have begun asking a counselor about psychedelic-assisted counseling advertised by a new local center. The counselor does not provide this service and wants to respond from an informed professional position without claiming expertise.

Which continuing education choice is most appropriate?

A) Attend the center's marketing seminar because local availability makes its information most relevant

B) Select evidence-focused education from qualified sources addressing current research, risks, limits, and counselor scope

C) Avoid all learning about the topic because the counselor does not currently offer the service

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Question 14

> An adult in rehabilitation counseling after a stroke uses supported communication because of mild expressive aphasia. The client's spouse wants treatment to focus on returning to work, but the client consistently identifies independent bus travel as the immediate goal.

Which initial planning response best supports collaboration?

A) Establish an accessible transit goal with the client and schedule later review of the work concern

B) Adopt return to work as the first goal because the spouse can describe daily functioning more easily

C) Postpone goal setting until the client can communicate without supported methods

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Question 15

A client repeatedly plans to complete a benefits application but postpones it whenever the paperwork feels confusing. The client has identified a trusted friend who is available on Saturday morning and wants a concrete plan.

Which counselor intervention best supports action?

A) Ask the client to visualize the disadvantages of never receiving the benefit.

B) Encourage the client to finish the entire application alone to strengthen self-efficacy.

C) Develop an implementation intention specifying when, where, and how the client will begin with the friend's support.

Question 16

A practice considers deploying a client-facing AI chatbot as a "24/7 counseling assistant." The vendor cannot explain how the bot handles crises, prevents fabricated advice, or protects conversation data. Clients would not initially be told that responses are automated.

What should the clinical director do?

A) Postpone deployment until privacy, validation, disclosure, human oversight, service limits, and crisis procedures are established

B) Deploy the bot only to established clients because the existing relationship supplies adequate oversight

C) Deploy the bot with a disclaimer that the practice is not responsible for automated responses

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Question 17

An adult with a mild intellectual disability arrives with a sister, who says, "I always sign things for him." No documentation identifies the sister as a legal decision-maker. The client communicates preferences clearly but needs extra time and simplified wording to understand written material.

How should the counselor handle informed consent?

A) Have the sister sign immediately because an intellectual disability eliminates the client's ability to consent

B) Ask the sister to leave and use the standard form without any communication adjustments

C) Provide accessible explanations, assess the client's understanding and decision-making ability, and verify whether anyone has legal authority before involving a substitute decision-maker

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Question 18

A client has a recurring nightmare after a frightening event. The counselor is trained in imagery rehearsal, and the client is stable, interested in the method, and able to use imagery without becoming overwhelmed.

How should the counselor apply the intervention?

A) Have the client repeatedly imagine the original nightmare without changing it until fear disappears.

B) Help the client create a less distressing ending and rehearse the revised dream imagery while awake.

C) Interpret each image as a symbol of an unresolved unconscious conflict.

Question 19

When asked why work has become difficult, a client provides excessive detail about several coworkers and office routines. The information remains logically connected, and the client eventually answers the question after repeated redirection.

Which mental status description best fits the client's thought process?

A) Flight of ideas

B) Circumstantial

C) Thought blocking

Question 20

At an initial appointment, a client says, “I’m not sure what I’m supposed to do here. Are you going to tell me how to fix my life?”

What is the counselor’s best response?

A) Explain that counseling is collaborative, describe how the counselor and client may work together, and ask what kind of help the client expected.

B) Reassure the client that the counselor will provide clear solutions once enough background information has been collected.

C) Begin asking about symptoms because uncertainty usually decreases after the formal assessment is completed.

Question 21

A board consent order permits a counselor to practice only with monthly supervision and requires written notice to the counselor's employer. The counselor has begun a new job but plans to wait until the employer asks about restrictions.

What should the counselor do?

A) Delay disclosure because the order is personal information unless a client is harmed

B) Give the required notice and ensure that compliant supervision is in place before continuing practice

C) Arrange informal peer consultation without telling the employer about the board order

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Question 22

A client with perfectionistic beliefs predicts, “If I ask a coworker to clarify one instruction, everyone will think I’m incompetent.” The client agrees to test the prediction in a low-risk situation.

What should the counselor do next?

A) Collaboratively specify the experiment, the predicted outcome, and what observations the client will record.

B) Assure the client that competent employees ask questions, making an experiment unnecessary.

C) Ask the client to request help with the most consequential work task to produce a decisive result.

Question 23

A counselor plans to use a visually presented career inventory with a client who has low vision. The counselor is trained in the inventory but does not know whether reading the items aloud would preserve standardization or whether an accessible form has been validated.

What should the counselor do?

A) Read every item aloud because using the same words preserves the validity of any standardized instrument

B) Consult authoritative instrument guidance or an assessment specialist and use a supported accommodation or suitable alternative

C) Administer the original visual form and interpret the results cautiously if the client misses any items

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Question 24

A routine telehealth session disconnects during discussion of a nonurgent work concern. The telehealth agreement designates a telephone call as the backup method and requires the counselor to reconfirm privacy before continuing.

What should the counselor do?

A) Use the agreed backup, confirm privacy, and continue or reschedule as appropriate

B) Send a direct message through the client's public social-media account because it is immediately available

C) Take no action because clients alone are responsible for restoring a failed connection

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Question 25

A counselor will meet Renee after she completes a six-week residential substance-use program. The discharge summary describes abstinence as the treatment goal, while the counselor's pre-admission notes document Renee's preference for a harm-reduction approach. No current outpatient goal has been confirmed.

What is the best preparation for the first post-discharge session?

A) Adopt the discharge goal because the residential program had the most recent contact with Renee

B) Review both records, note the difference, and prepare to ask Renee about her current goals and transition needs

C) Disregard the discharge summary because the outpatient record established Renee's original preference

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Question 26

A single parent who works rotating overnight shifts requests counseling at a community agency. The parent has missed appointments elsewhere because available times conflicted with work and child care. This agency offers some evening sessions and limited on-site child care.

What is the most appropriate intake response?

A) Refer the client elsewhere because prior missed appointments indicate poor readiness for counseling

B) Explore scheduling and child-care barriers with the client and determine whether the agency's available supports make services workable

C) Assign a standard daytime appointment so the counselor can evaluate commitment before discussing alternatives

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Question 27

A family court refers a parent and child for a specialized reunification service. The court's written order requires a provider with a particular reunification credential. A counselor has general family counseling experience but lacks that credential and specialized training.

What should the counselor do?

A) Accept the case because general family counseling experience covers the required service

B) Explain the limitation, refer to a qualified provider, and pursue specialized preparation only for future cases

C) Begin services while reading the credentialing standards between sessions

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Question 28

A primary care social worker referred Tasha for "anger management." During intake, Tasha describes caregiver strain and says her goals are to obtain respite and communicate more effectively with her father. Tasha has authorized the counselor to send the social worker a brief summary of engagement and goals, but not session notes.

What should the counselor communicate to the social worker?

A) Tasha's identified concerns and goals, along with any relevant coordination need permitted by the authorization

B) Confirmation that Tasha has an anger problem because that was the stated reason for referral

C) The complete intake narrative so the social worker can determine which account is more accurate

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Question 29

In an integrated care clinic, a client with diabetes has difficulty following a treatment plan. The counselor notices becoming irritated and lecturing the client, whose behavior reminds the counselor of a parent who rejected medical advice.

How should the counselor address this pattern?

A) Provide more detailed health education so the client understands the consequences of nonadherence

B) Identify and regulate the personal reaction, review it in supervision, and restore curiosity about the client's barriers

C) Ask the medical provider to take over counseling because frustration means the alliance cannot be repaired

Question 30

A graduate counseling intern is told to wear a badge reading "Licensed Professional Counselor" so clients will feel more confident. State law reserves that title for licensees, and the training site requires interns to identify their supervised status.

What should the intern and site do?

A) Use the licensed title because the supervisor assumes responsibility for the intern's services

B) Use the licensed title only with clients who already know the intern is a student

C) Use an accurate intern title and clearly identify the qualified supervisor and supervised status

Question 31

How are client strengths used most effectively in a treatment plan?

A) By documenting positive qualities separately from goals so they do not influence clinical decisions

B) By translating demonstrated abilities and available supports into resources for attaining objectives

C) By treating strengths as evidence that counseling barriers will resolve without specific planning

Question 32

Darius, a single father seeking help for gambling, wants a secular support group that he can reach by bus and attend while his child is in supervised care. The counselor has several recently verified programs in the referral directory.

How should the counselor narrow the referral options?

A) Recommend the closest program because distance is the most objective measure of access

B) Compare the verified programs with Darius's stated criteria and let him choose among the feasible options

C) Select a program based on which participant demographics seem most similar to Darius's background

Question 33

A client authorizes the clinic to send the client's medical record to a primary care physician for care coordination. The clinic maintains the counselor's private psychotherapy notes separately, and policy requires a distinct, specific authorization before those notes may be disclosed.

What should the counselor send?

A) The entire file, including the separate notes, because the physician is involved in treatment

B) The authorized relevant record, excluding separate notes unless specific authorization is obtained

C) Only appointment dates because no clinical information may be shared with another treating professional

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Question 34

A current client files an ethics complaint against a counselor. The counselor is angry and considers stating that counseling will continue only if the client withdraws the complaint. The client has not threatened the counselor or disrupted sessions.

What should the counselor do?

A) Avoid retaliation and base any care transition on clinical and ethical factors

B) Make withdrawal of the complaint a condition of treatment because trust has been damaged

C) End services immediately without referrals so the counselor does not influence the investigation

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Question 35

A person contacts a counseling-only clinic requesting evaluation and management of psychiatric medication. The clinic has no prescriber but can provide counseling and maintains referral relationships with medical providers.

What is the most appropriate intake action?

A) Explain the clinic's scope, check for urgent concerns and interest in counseling, and offer an appropriate prescribing referral

B) Schedule the person with a counselor and imply that the counselor can adjust medication after the first session

C) Reject the request without assessing urgency or explaining available referral and counseling options

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Question 36

What does a score on a criterion-referenced assessment primarily indicate?

A) Performance in relation to defined knowledge or skill standards

B) Rank compared with a representative norm group

C) Consistency of scores across different raters

Question 37

A client diagnosed with persistent depressive disorder says, “Persistent must mean permanent, so goals will not matter.” The client is considering ending counseling before the initial plan is completed.

How should the counselor address the diagnosis in treatment planning?

A) Avoid the term in future sessions so the client does not become discouraged by diagnostic language

B) Emphasize that the diagnosis is correct and proceed with goals even if the client remains doubtful

C) Clarify what the diagnosis describes, correct the permanence assumption, and invite questions while setting goals together

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Question 38

A former client sends a written demand alleging harm and requesting payment. The counselor's professional liability policy requires prompt notice of any potential claim and instructs the insured not to admit liability or alter records before receiving guidance.

What should the counselor do?

A) Pay the requested amount personally so no insurance record is created

B) Notify the carrier promptly, preserve the record, and follow appropriate legal and claims guidance

C) Revise the record to add details that make the counselor's decisions appear more defensible

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Question 39

A client is ordered to attend counseling as a condition of probation. The order requires the counselor to report monthly attendance and whether the client participates, but it does not authorize disclosure of session content.

How should the counselor address confidentiality at intake?

A) Promise complete confidentiality because the probation officer is not part of treatment

B) Explain the mandated status and reporting limits, obtain informed consent, and disclose only the required information

C) Tell the client that the complete record will be sent monthly because court involvement removes confidentiality

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Question 40

A counselor realizes that personal beliefs are creating discomfort while working with a same-sex couple. The counselor is otherwise trained to address the couple's presenting concern, and no competence-based reason for referral exists.

What is the counselor's most appropriate response?

A) Refer the couple immediately because personal-value conflicts always justify ending services

B) Seek consultation to manage personal values and continue competent, nondiscriminatory counseling

C) Explain the counselor's disapproval so the couple can decide whether to remain in counseling

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Question 41

> With two minutes remaining, a client says, “There’s something important I haven’t told you about my marriage.” The client states there is no immediate safety concern but does not want the issue ignored.

What should the counselor do?

A) Extend the session until the client has fully discussed the issue, regardless of later appointments.

B) Ask the client to wait until the next session because beginning a new topic near closing is inappropriate.

C) Acknowledge the importance, clarify enough to support containment, and agree on how to return to it promptly.

Question 42

> A 7-year-old receiving counseling after the death of a grandparent has emerging literacy skills. The current plan asks the child to write three paragraphs about emotions each week, but the child engages readily through drawing and storytelling.

Which revision best fits the child's developmental needs?

A) Replace the writing task with drawing or feeling choices followed by a brief child-led story

B) Keep the writing task so the child learns to express grief in an age-independent format

C) Ask the caregiver to write the paragraphs because adult language is more clinically useful

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Question 43

> A client accepts a referral to a pain-management specialist but declines to authorize communication between providers. The client still wants help preparing for the consultation and understands that the counselor cannot coordinate directly without permission.

Which response best supports care continuity while respecting the client's decision?

A) Contact the specialist with only diagnostic information because limited coordination does not require permission

B) Provide referral details and help the client prepare a concise summary the client may choose to share

C) Withdraw the referral because continuity is possible only when providers communicate with one another

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Question 44

A multilingual middle-school student is referred for possible learning difficulties after one year of instruction in English. The school proposes using only an English cognitive test that lacks appropriate norms for students with the learner's language history.

Which assessment approach is most appropriate?

A) Use multiple methods and qualified language support while considering educational opportunity, language development, culture, and measures appropriate for the student's background

B) Use the English test alone because cognitive scores are unaffected by language proficiency or educational experience

C) Delay any assessment indefinitely until the student speaks English without an accent

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Question 45

A college student seeks assessment for social anxiety. The student describes difficulty interpreting social cues since early childhood, intense distress when routines change, longstanding sensory sensitivities, and repeated highly focused interests. Anxiety increased only after peers began criticizing these behaviors.

Which assessment plan is most appropriate?

A) Diagnose social anxiety disorder solely because the student now fears negative evaluation

B) Exclude any neurodevelopmental condition because the student was admitted to college

C) Obtain a detailed developmental history and assess both autism-spectrum features and anxiety before determining the formulation

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Question 46

A billing review shows that both a primary and a secondary insurer paid the full allowed amount for the same sessions because coordination-of-benefits information was entered incorrectly. The resulting credit remains in the client's account.

What should the counselor or practice do?

A) Apply the credit to future sessions without informing either insurer

B) Correct the coordination error, notify the appropriate parties, and return any overpayment as required

C) Divide the extra payment between the client and the practice because both plans approved the claims

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Question 47

A counselor discovers that a progress note for Client A was saved in Client B's electronic chart. Clinic policy provides a controlled correction process, requires preservation of the audit trail, and directs staff to assess whether unauthorized access occurred.

What should the counselor do?

A) Restrict access as needed, report the error, and use the approved process to correct both records while preserving the audit trail

B) Copy the note into Client A's chart and leave the original in place to avoid altering a signed record

C) Permanently delete the misplaced note and avoid reporting it if no one admits reading it

Question 48

Which approach provides the most useful evaluation of a counselor's professional strengths and limitations?

A) Rely on confidence because discomfort usually indicates lack of competence

B) Use years of experience as the primary measure because skill increases predictably over time

C) Compare self-reflection with supervision feedback, client-relevant outcomes, and observed performance

Question 49

A client completing treatment for gambling disorder identifies the start of professional football season as a period of increased urges. The client already uses voluntary self-exclusion, bank transaction blocks, and weekly financial accountability with a trusted partner.

Which maintenance plan best strengthens relapse prevention?

A) Remove the transaction blocks gradually to determine whether the client can now gamble in moderation

B) Continue the safeguards and define a same-day support and treatment contact plan for strong urges or any bet

C) Rely on the partner to detect gambling activity before deciding whether additional support is necessary

Question 50

A county counseling program asks a counselor to help evaluate an artificial intelligence tool that predicts which clients are likely to miss appointments or deteriorate. The counselor has no training in algorithmic bias or the limits of predictive systems.

Which educational step is most appropriate before the counselor recommends whether to use the tool?

A) Learn the vendor's recommended settings because consistent configuration prevents unfair predictions

B) Complete prompt-writing training so the counselor can improve individual risk scores manually

C) Obtain independent education on validation, bias, privacy, human oversight, and effects on the populations served

Question 51

> During a session, a client says that the counselor has not been listening. The counselor's heart rate increases, and the counselor feels an immediate urge to defend the treatment plan and list everything remembered from prior sessions.

Which response best demonstrates self-regulation?

A) Pause, notice the defensive reaction, invite the client's perspective, and reflect on the interaction afterward

B) Explain the treatment plan in greater detail so the client can see that the counselor has been attentive

C) End the session early because strong internal reactions make productive discussion impossible

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Question 52

> A client reports intermittent suicidal thoughts without current intent or a plan. After a full risk evaluation, the counselor determines that outpatient care with a collaborative safety plan is appropriate under agency procedures. The client is willing to participate.

Which intervention should the counselor use?

A) Ask the client to sign a promise that no self-harm will occur before the next session.

B) Identify warning signs, internal coping steps, supportive contacts, professional resources, and ways to reduce access to lethal means.

C) Provide a crisis-line number and instruct the client to use it if the thoughts become more frequent.

Question 53

> A client signed an authorization allowing a treatment summary to be sent to a university program. The counselor did not send it before the authorization expired yesterday. Clinic policy requires a current written authorization for this disclosure.

What should the counselor do?

A) Send the summary because the authorization was valid when the client signed it

B) Change the date on the authorization after confirming the client's wishes by telephone

C) Obtain a new valid authorization before releasing the summary

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Question 54

> Nolan has low vision and tells his counselor that he sometimes skips an evening medication because two pharmacy bottles have nearly identical small-print labels. He reports no adverse symptoms. A written authorization permits coordination with the pharmacy and prescriber.

Which action is most appropriate for the counselor?

A) Encourage Nolan to rely on the slight cap-color difference and revisit the issue if an error occurs

B) Suggest taking both medications in the morning so Nolan does not need to identify them later

C) Document the access barrier and help Nolan request verified accessible labeling or packaging through the pharmacy and prescriber

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Question 55

> A college student with attention difficulties repeatedly intends to start assignments after dinner but loses track of time while gaming. The student wants a practical strategy and has identified 7:00 p.m. as a realistic start time.

Which intervention best supports follow-through?

A) Create an external cue at 7:00 p.m. and rehearse a specific sequence for pausing the game and opening the assignment.

B) Ask the student to rely on motivation and begin whenever the desire to study becomes strong enough.

C) Explore whether procrastination represents unconscious resistance to academic expectations.

Question 56

> At a multisite agency, a counselor is assigned a prospective client who is the plaintiff in an active lawsuit against the counselor's spouse. The counselor has followed the dispute closely, and another qualified clinician is available at a nearby site.

Which action best addresses counselor-client fit?

A) Accept the client but avoid discussing any topic related to the lawsuit

B) Ask the client to decide whether the counselor's knowledge of the dispute is a problem

C) Consult appropriately and arrange transfer to the available clinician because the direct conflict may impair objectivity

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Question 57

A client reports hearing accusatory voices and feeling watched during periods of methamphetamine use. The symptoms began after heavy use and have repeatedly resolved after sustained abstinence. The client has no known psychotic symptoms before substance use.

Which diagnostic assessment is most appropriate?

A) Diagnose a primary schizophrenia-spectrum disorder without examining the timing of substance exposure

B) Clarify the course of symptoms during intoxication, withdrawal, and abstinence before distinguishing a substance-induced condition from a primary psychotic disorder

C) Rule out all psychotic disorders because the client recognizes that methamphetamine can affect perception

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Question 58

In couple counseling, one bilingual partner pauses while searching for words in English. The other partner repeatedly answers on the person's behalf. The quieter partner looks frustrated but says nothing.

What is the counselor's best response?

A) Allow the more fluent partner to interpret because this preserves the couple's natural communication pattern.

B) Ask the quieter partner to respond only in English so each person's account can be evaluated consistently.

C) Slow the pace, ask each partner to speak for themself, and explore the quieter partner's preferred language support.

Question 59

A client says, "You write throughout the whole session. I can't tell whether you're listening to me or filling out a form." The counselor's notes are brief and can be taken less frequently.

How should the counselor respond?

A) Explain what the note-taking is for, invite the client's reaction, and collaborate on an adjustment that supports connection.

B) Stop all note-taking immediately and promise never to write during future sessions.

C) Reassure the client that professional note-taking does not interfere with attentive listening.

Question 60

Cronbach's alpha is most commonly used to estimate which characteristic of a multi-item assessment scale?

A) Predictive validity

B) Internal consistency

C) Interrater reliability

Question 61

A client with persistent worry spends hours each evening mentally reviewing possible problems. Attempts to suppress the thoughts make them return more strongly. The client wants a structured way to reduce the disruption.

Which intervention is most appropriate?

A) Encourage the client to answer every worry with evidence that the feared event cannot occur.

B) Establish a limited scheduled worry period and practice redirecting worry to that period when it appears earlier.

C) Ask the client to avoid all situations that tend to trigger uncertain thoughts.

Question 62

A counselor is leaving a group practice. The employment agreement and applicable law designate the practice as records custodian and require clients to receive notice of continuity options. The counselor wants to download all client charts to a personal drive for future reference.

What should the counselor do?

A) Download the charts because the counselor created most of the clinical content

B) Follow the custody and notice requirements and access or transfer records only through authorized processes

C) Delete the charts from the practice system after notifying clients of the departure

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Question 63

After describing a relapse in gambling, a client watches the counselor closely and asks, “Are you disappointed in me?” The counselor does not feel disappointed and is unsure what the client observed.

Which response would be most therapeutic?

A) “You’re wondering whether I’m disappointed. What did you notice from me that led you to ask?”

B) “No, counselors are trained not to judge clients for relapses.”

C) “The important issue is how you feel about the relapse, not how I feel about it.”

Question 64

A counselor who previously aged out of foster care is working with a young adult preparing to leave foster care. After hearing about the client's housing uncertainty, the counselor feels a strong urge to provide a personal telephone number, money, and a temporary room at home.

What is the most appropriate response to the counselor's reaction?

A) Describe the counselor's foster-care history and let the client choose which personal help to accept

B) Transfer the client immediately because shared life experience always prevents professional objectivity

C) Examine the rescue impulse in supervision and help the client access appropriate supports without creating personal dependency

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Question 65

What does a larger standard error of measurement indicate about an individual test score?

A) The score has stronger content validity

B) The score represents a higher percentile rank

C) The score is estimated with less precision

Question 66

Which information should have the greatest influence when selecting a counseling modality for a treatment plan?

A) The client's needs, preferences, access, relevant evidence, and any contraindications

B) The modality most commonly paired with the client's diagnostic label in agency templates

C) The approach the counselor uses most often, regardless of the client's circumstances

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Question 67

Avery, a high school student in a military family, will move to another state in three weeks. Avery and a parent have authorized coordination with the receiving school counselor. Avery wants continued support for lunchtime anxiety and values being able to visit the counseling office briefly when overwhelmed.

Which coordination action best supports the school transition?

A) Forward the entire school counseling file without discussing which supports Avery wants continued

B) Wait until after the move so the receiving counselor can assess Avery without prior expectations

C) Prepare a concise handoff of effective supports, Avery's preferences, and pending needs, and arrange an introductory contact

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Question 68

> Ten minutes into an initial interview, a client asks the counselor to provide a definitive diagnosis and a detailed treatment plan. Only the presenting concern and basic contact information have been discussed.

What is the counselor's best response?

A) Explain the purpose and sequence of the intake, clarify the client's expectations, and gather sufficient biopsychosocial information before offering a formulation

B) Select a diagnosis from the presenting concern so the client does not feel disappointed

C) Begin a detailed intervention plan without assessing history, functioning, strengths, or risks

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Question 69

> In family counseling, two caregivers want their 16-year-old to stop all online gaming. The adolescent wants fewer arguments and says gaming is the main way of maintaining friendships. All agree that meals have become a frequent conflict point.

Which initial goal best reflects collaborative prioritization?

A) Establish a shared trial of device-free meals while continuing to assess gaming, friendships, and family conflict

B) Adopt complete gaming abstinence because the caregivers hold responsibility for household rules

C) Focus only on the adolescent's friendship needs because treatment goals must belong to the youngest family member

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Question 70

> A 16-year-old independently consents to counseling under a state law that also gives the adolescent control over disclosure of those records. A parent demands detailed session notes, and no safety exception or court order applies.

What should the counselor do?

A) Release the notes because parents always control a minor's counseling information

B) Give the parent a verbal summary because oral disclosure is not a release of records

C) Follow the adolescent-consent law, maintain confidentiality, and discuss permissible family involvement with the client

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Question 71

> Within minutes of an initial appointment, a client begins describing a traumatic event in graphic detail and becomes visibly overwhelmed. The agency's intake normally starts with current needs, safety, service expectations, and a broad history before detailed trauma exploration.

What is the counselor's best response?

A) Require the client to finish a complete trauma narrative so the intake record is comprehensive

B) Stop the client from mentioning trauma at any point during the intake

C) Acknowledge the distress, offer to slow the pace, and explain how the structured intake can gather necessary information without requiring a detailed narrative now

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Question 72

> A counselor discovers that an insurer's online directory lists an outdated office address and a specialty the counselor does not hold. Prospective clients have begun calling for that specialty.

What should the counselor do?

A) Leave the listing unchanged because the insurer, not the counselor, published it

B) Request prompt correction and give prospective clients accurate information in the meantime

C) Accept the specialty referrals as long as the clients sign a general informed-consent form

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Question 73

> A court-appointed executor requests billing records needed to settle a deceased client's estate. The applicable law treats the executor as the client's personal representative for relevant records, and the executor provides valid appointment documents.

How should the counselor respond?

A) Refuse because no information may ever be released after a client's death

B) Release the complete clinical record because an executor automatically needs every counseling detail

C) Verify the documents and provide the records relevant to the authorized estate purpose through a secure process

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Question 74

> An agency is receiving more requests from autistic adults seeking counseling for workplace stress. A counselor's prior education focused almost entirely on childhood deficit models and did not address adult or autism-affirming practice.

Which learning plan would best prepare the counselor to serve this population?

A) Adapt the childhood methods independently because the core features of autism remain the same across age groups

B) Study current adult and autism-affirming evidence, include autistic perspectives, and obtain qualified feedback on practice

C) Continue current methods until an autistic client reports that the approach is ineffective

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Question 75

> A counselor is trained to use an adult career-interest inventory. A middle school asks the counselor to administer it to a 13-year-old to determine eligibility for a technical education track. The counselor has not established that the instrument is appropriate for this age or purpose.

What is the best response?

A) Administer the inventory and note the student's age as a limitation in the report

B) Let the student's parent interpret the results because the parent knows the student's interests

C) Verify population and purpose suitability, consult as needed, and select an appropriate alternative if support is lacking

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Question 76

> A client practicing for a job interview asks for feedback. During rehearsal, the client gives thoughtful answers but speaks so softly that the counselor can barely hear them.

Which feedback is most useful?

A) “You need to project confidence if you want an interviewer to take you seriously.”

B) “Your interview skills will improve after you work through the anxiety underneath them.”

C) “Your answers were clear, and I had difficulty hearing the last two. Would you like to practice them at a stronger volume?”

Question 77

> A 16-year-old is referred after a school employee noticed cannabis vaping. A parent remains in the room and answers most substance-use questions. The adolescent gives brief answers and watches the parent before responding.

What is the most useful next assessment step?

A) Rely on the parent's report because an adolescent's self-report about substance use is inherently invalid

B) Order a comprehensive adult personality inventory before asking additional substance-use questions

C) Explain the limits of privacy, conduct an appropriate private portion of the interview, and combine an adolescent substance-use screen with follow-up questions

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Question 78

> An automated intake system routes applicants who use public insurance to a waitlist even when clinicians have matching openings. Practice policy prohibits payer-based discrimination and requires human review of consequential automated decisions.

What should the practice do?

A) Continue using the system because consistent automation is less biased than staff judgment

B) Keep the routing rule but add a notice that insurance type may affect scheduling

C) Suspend the discriminatory routing, review affected applications, and audit and correct the system before reuse

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Question 79

> In a counseling group, one member says, “I need extra time to put my thoughts into words.” Other members often finish the member’s sentences or move on during pauses. The member has begun participating less.

What is the leader’s best intervention?

A) Ask the member to prepare written statements before each meeting to keep the discussion moving.

B) Name the pattern, ask the member what would support participation, and invite the group to tolerate a slower pace.

C) Direct the other members to remain completely silent whenever this member begins speaking.

Question 80

When a client and counselor revise a treatment goal, which documentation best supports continuity of care?

A) Replace the original goal with the new goal so later readers see only the current plan

B) Record the change, the client's reason for it, the revised goal, and the related next step

C) Retain the original goal without revision until the client's diagnosis also changes

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Question 81

During a restoration drill, a clinic learns that its backup service has copied unencrypted counseling records into an unapproved consumer cloud account. Security policy requires encrypted backups in approved storage and immediate review of unauthorized repositories.

What should the clinic do?

A) Leave the backup in place because restoration testing proved that it works

B) Download another copy to a personal drive before anyone changes the account

C) Secure the repository, report and assess the exposure, and migrate backups to approved encrypted storage

---

Question 82

How is unconditional positive regard best expressed when a client describes behavior that harmed another person?

A) Maintain respect for the client while openly exploring the behavior, its effects, and responsibility for change.

B) Avoid discussing the harm so the client does not experience the counselor as judgmental.

C) Communicate approval of the behavior to demonstrate complete acceptance of the client.

Question 83

A transgender client seeks counseling for anxiety and says they are undecided about any social transition. A draft plan written by a previous provider lists beginning a social transition as a treatment goal, but the client does not identify that as an objective.

What is the most appropriate plan revision?

A) Retain the transition goal because it may eventually reduce anxiety even if the client is uncertain

B) Remove the presumed goal and collaborate on the client's anxiety priorities, exploring identity only as the client wishes

C) Replace the transition goal with an objective to decide on a transition before anxiety treatment begins

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Question 84

A counselor trained in exposure and response prevention is guiding a planned contamination exposure. The client asks repeatedly, “You know I won’t get sick from touching this, right?” The client and counselor previously agreed that seeking reassurance maintains the fear cycle.

How should the counselor respond?

A) Remind the client of the agreement and coach the client to notice anxiety without resolving the uncertainty.

B) Reassure the client once so anxiety falls enough to complete the exposure.

C) End the exposure because the reassurance requests show that the client is not ready for this method.

Question 85

> A counselor is considering adding neurofeedback after hearing enthusiastic reports from colleagues. The counselor has no training in the method and finds mixed claims about its uses and evidence base.

What is the best next professional development step?

A) Review current research and professional guidance, clarify training standards and scope, and seek qualified instruction before representing competence

B) Purchase the equipment and practice with selected clients while completing self-paced tutorials

C) Adopt the method for concerns described in colleague testimonials because peer experience is practice-based evidence

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Question 86

Which use of case conceptualization contributes most directly to treatment planning?

A) Compiling a complete chronological history without forming conclusions about what maintains the concern

B) Selecting interventions from the diagnostic label without considering current protective factors

C) Organizing contributing and protective factors into testable hypotheses that guide goals and interventions

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Question 87

> Two partners identify “communicate better” as a counseling goal. They currently have about five escalating arguments each week and agree that a short evening check-in is feasible.

Which objective best makes the goal measurable and attainable?

A) The partners will eliminate all disagreements during daily check-ins for four weeks

B) The partners will complete a 10-minute check-in four evenings weekly and track escalation for four weeks

C) The partners will hold check-ins when needed and discuss progress at the next four sessions

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Question 88

Which outcome best demonstrates that a referral loop has been closed?

A) The receiving service has the needed authorized information, responsibilities are clear, and the client has connected with care

B) The counselor has sent a referral form and documented the date, even though receipt has not been confirmed

C) The client has received several provider names and understands that arranging care is now solely the client's responsibility

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Question 89

> Three weeks after giving birth, a client reports recurrent images of accidentally dropping the baby. The images are unwanted and horrifying, and the client avoids carrying the baby near stairs. The client denies wanting to cause harm and recognizes that the images come from the client's own mind.

Which assessment approach is best?

A) Assess the images' frequency, meaning, associated rituals or avoidance, intent, insight, and any psychotic symptoms before reaching a diagnostic conclusion

B) Conclude that the client is experiencing psychosis because all thoughts involving infant harm are delusions

C) Reassure the client that unwanted thoughts never require further risk or diagnostic assessment

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Question 90

Which counselor statement best reflects a client's meaning rather than only content or feeling?

A) "Being the person everyone depends on seems closely tied to how you measure your worth."

B) "You handled the bills, scheduled the appointments, and drove everyone to work."

C) "You feel exhausted and frustrated by how much your family expects from you."

Question 91

> When a counselor announces a planned vacation and coverage arrangements, a client says the absence proves that everyone eventually abandons them. The counselor feels guilty and considers canceling the vacation to show commitment.

How should the counselor respond?

A) Maintain the planned boundary and coverage, explore the client's reaction therapeutically, and examine the counselor's guilt in supervision

B) Cancel the vacation because consistency requires the counselor to remain available when separation distress emerges

C) Tell the client that the reaction is irrational and should not be discussed further

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Question 92

> A client follows the counselor's public hiking account and comments frequently on personal photographs. The counselor feels flattered and is tempted to reply in a friendly way because the account is visible to anyone.

What is the most appropriate self-aware response?

A) Reflect on the desire for approval, maintain the established digital boundary, and discuss the contact in counseling if clinically relevant

B) Reply briefly because interaction on a public account cannot affect the counseling power difference

C) Block the client without discussion and terminate counseling to prevent any future online contact

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Question 93

> Mateo accepted a referral to a caregiver-support group that the counseling center's directory listed as Spanish-language. When he called, the program said the group is currently offered only in English, so he did not enroll.

How should the counselor respond to the failed linkage?

A) Verify the program change, update the directory, and help Mateo locate a currently verified Spanish-language option that fits his preferences

B) Encourage Mateo to attend the English-language group because the topic is more important than the language used

C) Keep the directory entry unchanged until the program revises its public website

---

Question 94

> A client wants a traditional healer to participate in planning care. The counselor has not worked with this healing tradition, and nothing described by the client suggests an immediate safety concern.

What is the best professional growth response?

A) Decline involvement because unfamiliar traditional practices cannot be integrated with counseling

B) Assume the traditional healer will replace counseling and begin planning for termination

C) Ask how the client wants the practice included and, with consent, seek culturally informed consultation before coordinating within competence

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Question 95

> A client with unpredictable seasonal income assumes that counseling is free. The clinic charges fees, has a late-cancellation policy, and offers a separate application for reduced fees. None of these terms has yet been reviewed with the client.

What should the counselor do before services begin?

A) Delay all discussion of cost until the client receives the first bill

B) Waive every fee without following the clinic's stated assistance process

C) Explain fees, cancellation terms, and available assistance clearly enough for the client to make an informed choice

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Question 96

> In a relapse-prevention group, Maya described a recent craving, practiced urge surfing, and planned to call her recovery sponsor if the craving returned. Other members also shared personal experiences during the session.

What should the counselor include in Maya's individual progress note?

A) A detailed summary of every member's disclosure to show how the group influenced Maya

B) Only the statement "attended and participated" because group content cannot be documented individually

C) Maya's relevant participation, response to the intervention, and follow-up plan without identifying other members' disclosures

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Question 97

> A parent schedules an intake for a 20-year-old child, completes the history form, and asks to receive updates after each session. The 20-year-old arrives independently and has not authorized disclosure to the parent.

What should the counselor do during intake?

A) Treat the parent as the client because the parent initiated contact and completed the form

B) Explain confidentiality to the adult client, verify the client's own account and preferences, and obtain authorization before sharing information with the parent

C) Send routine updates to the parent but omit only the details the counselor considers especially sensitive

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Question 98

> An adult in supported living repeatedly uses the staff call button when routine check-in times are unpredictable. The client can use a picture schedule and can make a direct request for attention when prompted. Staff report that immediate attention after repeated calls has unintentionally reinforced the pattern.

Which behavioral management plan best fits these facts?

A) Schedule predictable check-ins, teach the direct request, reinforce its use, and track call-button frequency

B) Remove the call button for increasing periods after repeated use and record whether the client complains

C) Ask each staff member to respond according to personal judgment so the client learns flexibility

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Question 99

A telehealth platform disconnects during a session with a client who has episodic panic attacks but no current acute safety concern. The counselor and client previously documented a backup telephone number, the client's current location, and local emergency options.

What should the counselor do to implement the disruption plan?

A) Use the agreed backup number, confirm the client's status, and decide whether to resume or reschedule

B) Wait for the client to reconnect so that the counselor does not cross a communication boundary

C) Contact emergency services immediately because any disconnection during mental health care is an emergency

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Question 100

Which feature makes a community referral most actionable for a client?

A) Current eligibility and availability are verified, the service fits the client's needs and preferences, and access steps are clear

B) The client receives a large directory containing every agency that has offered a related service in recent years

C) The counselor provides the name of a well-known agency and asks the client to investigate whether it is appropriate

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Question 101

A client comes to a community center several hours after learning that a sibling died unexpectedly. The client is physically safe, denies thoughts of self-harm, and says, “I feel nothing. Does that mean something is wrong with me?”

Which response is most appropriate?

A) Encourage the client to describe the death in detail so the expected emotions will emerge.

B) Explain that early reactions can include numbness, then ask what immediate support would feel most helpful.

C) Interpret the numbness as denial and emphasize the need to begin accepting the loss.

Question 102

An 11-year-old responds “I don’t know” to repeated questions about changing schools but becomes animated while arranging miniature figures on a table. The counselor is trained to use developmentally appropriate expressive activities.

What is the counselor’s best response?

A) Continue asking direct questions because verbal answers are necessary for meaningful counseling.

B) Interpret the figure placed far from the others as proof that the child feels abandoned.

C) Invite the child to show what school has been like with the figures and let the child explain the arrangement.

Question 103

During an office session, an agitated client stands, clenches both fists, moves close to the counselor, and shouts, “You’re going to listen now.” Agency protocol directs counselors to maintain access to an exit and summon trained assistance when behavior becomes threatening.

What should the counselor do?

A) Remain seated to appear nonthreatening and invite the client to explain the anger fully.

B) Create distance toward the accessible exit, state a clear behavioral limit in a calm voice, and summon assistance according to protocol.

C) Match the client’s volume and direct the client to sit down immediately.

Question 104

In family counseling, parents and their adult daughter disagree about her dating choices. The parents describe protecting family values, while the daughter describes wanting respect for her adulthood. Each asks the counselor to endorse one side.

What is the counselor’s best intervention?

A) Elicit what protection, respect, and family connection mean to each person and identify shared hopes before negotiating differences.

B) Explain that the adult daughter’s autonomy makes the parents’ cultural concerns irrelevant to counseling.

C) Support the parents’ position because family counseling should preserve existing family values.

Question 105

A client's insurance coverage will end in two weeks after a job change. The client is stable but needs ongoing counseling and agrees to transfer to a sliding-scale community clinic. The current counselor can provide two bridge sessions under agency policy.

Which plan best addresses the financial transition?

A) Use the final sessions to process the insurance loss and let the client contact clinics after coverage ends

B) Discharge now so the client can use the remaining coverage to pay the new clinic independently

C) Confirm an affordable intake, coordinate authorized information, and use bridge sessions and contingency resources as needed

Question 106

A bereaved client occasionally dreams of a deceased spouse and, on waking, briefly senses the spouse's comforting presence. The client says this experience is consistent with family and spiritual beliefs, does not command behavior, and causes no impairment or fear.

How should the counselor assess this report?

A) Record a psychotic disorder because sensing a deceased person is always a hallucination requiring diagnosis

B) Ignore the experience entirely because culturally meaningful experiences can never be clinically relevant

C) Explore the experience's cultural meaning, context, duration, distress, impairment, and related symptoms before deciding whether it is pathological

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Question 107

A group practice wants to engage a prelicensed associate as an independent contractor who will set fees and bill under the associate's own name. State rules permit associates to practice only as employees of approved sites under board-qualified supervision and prohibit independent billing.

What should the practice do?

A) Use the contractor arrangement if the associate meets weekly with any licensed clinician

B) Establish a rule-compliant employment and supervision arrangement or do not engage the associate for clinical work

C) Let the associate bill independently but add the supervisor's name to progress notes

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Question 108

A researcher compares results from a brief depression screener with diagnoses from an established structured diagnostic interview administered at the same time. This comparison primarily evaluates which property of the screener?

A) Concurrent criterion validity

B) Internal consistency

C) Test-retest reliability

Question 109

A college counseling center can provide only two more sessions to a student who needs longer-term trauma treatment. The student accepts a referral, but the receiving clinic's first appointment is six weeks away. Center policy permits brief bridge contacts during a documented transition.

Which plan best supports continuity during the wait?

A) Use both remaining sessions now and tell the student to seek emergency care if distress increases later

B) Ask the student to contact the receiving clinic independently and return after the six-week wait if needed

C) Confirm the referral, schedule permitted bridge contacts, and document coping and urgent-care options for the interim

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Question 110

In an acceptance-based intervention, what is the main purpose of cognitive defusion?

A) To replace each negative thought with a more objectively positive statement.

B) To identify the childhood event that originally produced the thought.

C) To help the client experience thoughts as mental events rather than commands or literal facts.

Question 111

One week after a motor vehicle crash, a client reports being startled by horns and briefly feeling as if the crash is happening again. The client is safe, functioning with support, and asks, “Am I going crazy?”

How should the counselor respond?

A) State that the symptoms confirm a trauma-related disorder and begin long-term trauma processing.

B) Advise avoiding traffic sounds until the reactions have completely stopped.

C) Normalize that such reactions can occur after a frightening event, teach present-orientation skills, and explain when additional help is warranted.

Question 112

A client with a traumatic brain injury has slowed information processing and difficulty retaining lengthy verbal instructions. A standard 90-minute, fast-paced counseling group is available, but the client prefers individual work and benefits from summaries and repetition.

Which modality plan is most responsive to the available information?

A) Enroll the client in the standard group and require detailed note-taking after each topic

B) Postpone counseling until processing speed and memory return to their previous levels

C) Use shorter individual sessions with paced content, accessible summaries, and checks for understanding

---

Question 113

A client has sent several late-night noncrisis messages to the counselor, although the informed-consent agreement states that messages are checked only during business hours. The client says the messages provide relief even without a reply.

What should the counselor do in session?

A) Continue allowing the messages because the client does not expect a response.

B) Validate the need for relief, restate the communication boundary, and develop acceptable between-session coping and contact options.

C) Block the client's number outside business hours so the limit is enforced consistently.

Question 114

A telehealth counselor now conducts eight sessions on most days with almost no time between appointments. The counselor notices fading concentration late in the day, checking the clock repeatedly, and needing increasing time to recover after work, although no known error has occurred.

What is the best response?

A) Add a brief relaxation exercise between sessions while keeping the schedule unchanged

B) Wait for a client complaint before treating the concentration change as a professional concern

C) Review capacity with a supervisor, adjust scheduling or workload, monitor care quality, and build a sustainable recovery plan

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Question 115

Which feature is the strongest indicator that a continuing education activity can support meaningful skill development?

A) It awards a large number of credit hours for independent reading

B) It includes clear objectives, qualified instruction, active practice, and feedback tied to current evidence

C) It is popular with counselors from many specialties and requires no preparation

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Question 116

A client whose apartment is heavily cluttered identifies inviting grandchildren to visit as a meaningful long-term goal. A whole-home cleanout feels overwhelming, but the client agrees that the blocked exit path is the most urgent area and believes one small section can be addressed at a time.

Which initial objective is most appropriate?

A) The client will clear the entire apartment with family help during the next available weekend

B) The client will clear and maintain one agreed section of the exit path within two weeks, then review the next step

C) The client will postpone all home-based objectives until the idea of a complete cleanout no longer causes distress

---

Question 117

> Nadine asks her counselor to coordinate with an elder from her cultural community as an additional source of support. She authorizes limited care-planning communication and says she wants to continue counseling while also participating in a ceremony the elder recommends.

What is the counselor's best coordination approach?

A) Decline collaboration because a nonclinical cultural support cannot contribute to a counseling plan

B) Ask the elder to direct the treatment plan so the services remain culturally consistent

C) Clarify with Nadine the roles and information she wants shared, then coordinate complementary supports within the authorization

---

Question 118

> A client considering whether to leave a family-owned business says that a trusted aunt has always helped with major decisions. The client wants the aunt's perspective included but does not want the aunt to decide the counseling goals.

How should the counselor reflect this preference in the plan?

A) Exclude the aunt because involving relatives prevents independent client decision-making

B) With the client's permission, define a limited consultative role for the aunt while keeping goals client-directed

C) Make family agreement the outcome criterion because the decision affects a family-owned business

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Question 119

> A counselor is assessing trauma symptoms in a 5-year-old whose caregiver prefers Spanish. The child has limited reading ability and expresses experiences mainly through play and short phrases.

Which assessment strategy is best?

A) Administer a self-report measure designed for English-speaking adults because adult instruments contain more detailed questions

B) Use only unstructured observation because no standardized information can be useful with a young child

C) Combine developmentally appropriate interview and observation with a validated caregiver measure available in the caregiver's preferred language

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Question 120

Jordan missed two counseling visits. The prior note states that rotating work shifts were creating scheduling problems and that Jordan chose text reminders. The scheduling log shows that reminders were sent, but it does not indicate why Jordan missed the visits. Another appointment is now scheduled.

How should the counselor prepare for the next meeting?

A) Review the attendance and contact information and prepare neutral questions about barriers and scheduling preferences

B) Add "resistant to treatment" to the case summary because the reminders did not improve attendance

C) Plan to restate the treatment goals without discussing the missed visits or the rotating shifts

Question 121

During an interpreted counseling session, a qualified interpreter says that a counseling term has no direct equivalent in the client's preferred language. What should the counselor do?

A) Ask the interpreter to select the closest single word without further discussion

B) Omit the concept so the difference in language does not interrupt the session

C) Explain the concept in plain, concrete language, allow the interpreter to convey the meaning, and check the client's understanding

Question 122

During intake, a counselor realizes that the prospective client is the mediator currently handling the counselor's own divorce. Other qualified counselors at the agency have openings, and the counselor is concerned that the existing role could affect objectivity.

What is the most appropriate fit decision?

A) Use appropriate internal consultation and arrange assignment to another qualified counselor while limiting disclosure of personal details

B) Accept the client and avoid mentioning the existing role unless the client identifies a problem

C) Accept the client because professional training should eliminate the effect of personal conflicts

Question 123

A telehealth client joins a scheduled session from the back seat of a rideshare vehicle. The driver can hear the conversation, and the client says there will be no private location for another 20 minutes. No urgent concern is present.

What is the counselor's best response?

A) Proceed because clients may waive privacy by choosing a public setting

B) Avoid clinical discussion until privacy can be established, then resume or reschedule according to policy

C) Ask the driver to sign a confidentiality statement and continue the session immediately

Question 124

A client describes coercive control by a partner but denies immediate danger and does not want to leave the relationship today. The client wants to feel more prepared if the situation worsens.

Which response best supports safety and empowerment?

A) Tell the client that leaving immediately is the only responsible choice.

B) Respect the client's decision while collaboratively developing a personalized safety plan and reviewing confidential support options.

C) Recommend couple counseling so both partners can negotiate rules for the relationship.

Question 125

For more than a year, a client has experienced intrusive fears of contaminating family members and washes for about three hours each day to reduce anxiety. The client recognizes that the fears are excessive but feels unable to resist the rituals. The symptoms substantially disrupt work and family life.

Which diagnosis is best supported by the information?

A) Illness anxiety disorder

B) Obsessive-compulsive disorder

C) Delusional disorder, somatic type

Question 126

A clinic's record system repeatedly displays transgender clients' former names on appointment screens even after staff update demographic fields. Several clients report avoiding the waiting room because staff call the wrong name.

Which response best integrates professional development and advocacy?

A) Teach affected clients how to remind staff of the correct name at every visit

B) Obtain affirming-practice training, examine the workflow with relevant client input, and advocate for a system correction

C) Recommend separate appointment times for transgender clients until the software is replaced

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Question 127

> A prospective client asks a counseling clinic for a psychological hardship evaluation to support an immigration petition. The clinic provides treatment but does not conduct forensic or immigration evaluations, and its reports are not designed for legal proceedings.

What is the best intake response?

A) Explain the clinic's treatment role and limits, assess whether the person also seeks counseling, and offer referral information for a qualified independent evaluator

B) Begin therapy and promise that the treating counselor will later write whatever legal opinion the client needs

C) Decline all contact without explaining the service mismatch or offering relevant referral options

---

Question 128

> A vocational rehabilitation counselor is helping a veteran compare training programs. The counselor is competent in career counseling but is uncertain how two federal education benefits would interact. An accredited veterans' benefits specialist is available for case consultation.

Which professional development response best fits this situation?

A) Obtain targeted consultation from the benefits specialist while retaining responsibility for the career counseling

B) Enter clinical supervision focused on counseling skills before discussing any training options with the client

C) Transfer the entire case to the benefits specialist because one factual uncertainty makes the counselor incompetent

---

Question 129

How should a counselor generally treat a deceased client's confidential information?

A) Continue to protect it and disclose only when valid consent, lawful authority, or another applicable exception permits

B) Release it to any family member because confidentiality ends automatically at the client's death

C) Destroy it immediately so no personal representative can request access

---

Question 130

Which device-management practice best supports compliant electronic counseling work?

A) Use supported devices, timely security updates, approved encryption, and access controls such as multifactor authentication

B) Continue using unsupported software if the device has never displayed a visible security warning

C) Share one clinic login among staff so urgent work is not delayed by individual authentication

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Question 131

A client reports that a former partner has been following the client to work, recently threatened to break into the home, and previously strangled the client. The former partner owns a firearm and has ignored a request to stop contact. The client is afraid to return home tonight.

What is the counselor's best assessment response?

A) Conduct an immediate danger assessment and collaborate on urgent safety and support options appropriate to the level of risk

B) Focus only on whether the client meets criteria for an anxiety disorder before discussing the former partner's behavior

C) Recommend a routine communication exercise with the former partner to clarify expectations

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Question 132

Ms. Patel lives alone and has authorized her counselor to coordinate with a meal-delivery program. The program reports that several meals were returned unopened. Ms. Patel says new dental pain makes solid food difficult to eat and emphasizes that she wants to remain in her home.

Which coordination plan best reflects the available information?

A) Tell the program that Ms. Patel is refusing meals and needs closer compliance monitoring

B) With Ms. Patel's input, relay the functional barrier and coordinate temporary texture-appropriate meals and access to dental support

C) Recommend an assisted-living placement because returning meals indicates that independent living is no longer feasible

---

Question 133

A client advisory group reports that a community agency's orientation materials are hard to read and that translated versions use confusing technical language. Staff members have not received training in plain-language or culturally responsive communication.

Which action best combines professional learning and client advocacy?

A) Seek relevant training and collaborate with clients and qualified language resources to revise and test the materials

B) Keep the materials unchanged and remind clients that staff can answer questions during orientation

C) Shorten the materials by deleting explanations that require translation or literacy review

---

Question 134

> Luis, an adult with a mild intellectual disability, is considering supported-housing options. He has authorized coordination with his sister, an independent-living advocate, and a case manager. During planning calls, his sister answers questions quickly, and Luis stops participating. Luis tells the counselor that he wants more time to answer for himself.

How should the counselor prepare for the next coordination meeting?

A) Ask the sister to present Luis's preferences because she can communicate them more efficiently

B) Exclude the other participants so Luis will not experience pressure during planning

C) Confirm Luis's preferred meeting format and structure the meeting to give him response time before others contribute

---

Question 135

> A client receiving counseling for panic symptoms is newly diagnosed with diabetes. The medical provider explains that several episodes of sweating, shaking, and confusion were caused by low blood glucose, although the client also continues to experience panic attacks in other situations. The client consents to care coordination.

How should the counselor revise the treatment plan?

A) Remove all anxiety goals because the physical symptoms now have a medical explanation

B) Continue the original plan unchanged because medical information is outside counseling treatment planning

C) Coordinate relevant care and help the client distinguish episodes while retaining appropriate panic-related goals

---

Question 136

Which task is most appropriate for a counselor to delegate to a trained, nonlicensed administrative assistant?

A) Scheduling appointments using privacy-protective procedures without making clinical decisions

B) Conducting an intake risk assessment from a script and deciding whether urgent care is needed

C) Interpreting screening results and explaining a provisional diagnosis to a client

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Question 137

Which evidence most strongly suggests that continuing education has improved a counselor's competence?

A) Demonstrated skill improvement with appropriate feedback and favorable client-relevant process or outcome data

B) Increased confidence immediately after attending a well-reviewed presentation

C) Completion of more credit hours than the licensing jurisdiction requires

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Question 138

In a small community where incidental contact with clients is reasonably foreseeable, which boundary practice is most appropriate?

A) Discuss foreseeable contact, agree on privacy-protective boundaries, document them, and monitor their effect

B) Terminate counseling whenever a counselor and client might encounter each other outside the office

C) Ignore incidental contact because boundary concerns arise only from intentional social relationships

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Question 139

A client scores above the recommended cutoff on a well-validated screening tool. What is the most accurate interpretation?

A) The client definitively meets diagnostic criteria for the disorder

B) The result indicates that further clinical assessment is warranted

C) The result proves that treatment must begin before an interview occurs

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Question 140

> A client with opioid use disorder is preparing to leave residential treatment. The client plans to continue prescribed buprenorphine, has a supportive sister, and wants outpatient counseling near home. Reduced opioid tolerance increases overdose risk after discharge.

Which aftercare plan is most appropriate?

A) Confirm medication and counseling appointments, include overdose-prevention resources, and define support and reentry steps

B) End buprenorphine at discharge so the counselor can evaluate recovery without medication

C) Give the client general program numbers and wait for the client to request help after returning home

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Question 141

> An adult who uses an augmentative and alternative communication device arrives with a support worker. The counselor directs each question to the worker and moves to the next topic before the client has time to compose responses.

How should the counselor modify the intake?

A) Continue using the worker as the primary respondent because device-assisted answers take longer

B) Ask the worker to decide which topics the client is capable of discussing

C) Address the client directly, ask about communication preferences, allow sufficient response time, and involve the worker only as authorized

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Question 142

A community agency requires outreach after two missed appointments and closes a case after 30 days without client contact. Twenty days after the last visit, a client calls, explains that phone service was disconnected, and asks to resume counseling. The client reports no acute safety concern.

Which response best supports continuity within the stated policy?

A) Close the case because two missed appointments demonstrate that the client is not ready to participate

B) Review the access barrier, update backup contact and attendance plans, and reschedule while the case remains open

C) Keep the case open indefinitely without revisiting communication plans because the client has now made contact

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Question 143

Which practice is most appropriate when a counselor may use a collection agency for overdue balances?

A) Give the agency unrestricted record access so it can decide what information will encourage payment

B) Avoid mentioning collections until a client misses payment because advance notice may discourage treatment

C) Disclose the collection policy in the financial agreement and release only legally permitted information if collection becomes necessary

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Question 144

Before signing a templated progress note, a counselor notices that an automatically carried-forward field says "employment unchanged." During the session, the client reported starting a job and changed future appointment times to fit the new schedule.

What should the counselor do?

A) Correct the field with the individualized information and verify the other carried-forward content before signing

B) Sign the note as generated and add a correction only if the employment change later affects treatment

C) Keep the standardized field because consistency across notes is more important than a nonclinical detail

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Question 145

A treatment team determines that weekly outpatient counseling is no longer sufficient for a client with rapidly escalating manic symptoms, severe sleep loss, and major functional impairment. The client agrees to an urgent evaluation for a higher level of care.

What should the outpatient counselor do to support continuity?

A) Increase between-session assignments while waiting until the next routine appointment to review symptoms

B) End outpatient services immediately so the higher-level program assumes responsibility without overlap

C) Coordinate the urgent evaluation, transfer authorized information, and maintain a bridge until care is accepted

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Question 146

What is the central purpose of blocking in a counseling group?

A) To prevent members from discussing topics that may produce strong emotion.

B) To interrupt behavior that impairs safety or productive participation while keeping the member engaged.

C) To stop a member whenever the leader disagrees with the member's perspective.

Question 147

> A community college student seeks help for test anxiety and avoidance. The student works two jobs and cannot afford weekly private counseling, but an evening campus skills group is free, and a trusted classmate already supports study routines.

Which plan best integrates the student's strengths and barriers?

A) Recommend weekly private counseling because individual treatment permits the most personalized plan

B) Delay treatment until the student can reduce work hours and attend daytime appointments consistently

C) Use the accessible evening group and, with the student's agreement, incorporate peer accountability into objectives

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Question 148

> A community outreach counselor repeatedly helps families apply for housing that remains unavailable. The counselor has become cynical, catches themself blaming clients for incomplete applications, and feels guilty about being unable to change the housing shortage.

Which response best addresses the counselor's current functioning?

A) Use supervision and team support to examine the loss of empathy, set sustainable limits, and identify realistic resource or advocacy actions

B) Lower emotional involvement with all clients so that unsuccessful applications no longer affect the counselor

C) Work additional unpaid hours on applications to compensate for the shortage of community resources

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Question 149

After immediate physical safety has been established, which principle generally guides early psychological crisis support?

A) Offer practical assistance, connection, and coping support without requiring detailed emotional disclosure.

B) Encourage a complete retelling of the event before discussing present needs.

C) Delay supportive intervention until the person displays a predictable emotional reaction.

Question 150

> One partner says, “When I talk about a hard day, I want understanding.” The other replies, “I offer solutions because doing nothing feels uncaring.” Their conversations usually end with both feeling rejected.

Which in-session intervention is most appropriate?

A) Ask each partner to prepare a list of preferred solutions before discussing another stressful event.

B) Coach one partner to describe the experience while the other reflects the feeling and asks whether empathy or problem solving is wanted.

C) Advise the solution-focused partner to stop offering suggestions during all future conversations.

Question 151

> During a group session, one member makes a demeaning comment about immigrants. Another member becomes tense and silent, while several others look toward the leader.

What is the leader’s best immediate intervention?

A) Wait to see whether group members challenge the comment without leader influence.

B) Move to a less controversial topic and speak privately with both members after the session.

C) Interrupt the comment, name its potential impact, reaffirm group expectations, and invite affected members to choose whether to respond.

Question 152

What is the primary purpose of linking in group counseling?

A) To redirect a member whose comments are unrelated to the group’s stated topic.

B) To summarize the leader’s interpretation before the group moves to another issue.

C) To connect members with shared themes so they can respond directly to one another.

Question 153

> A client reports injecting illicit fentanyl daily, surviving an overdose two days ago, and planning to use alone again tonight. The client currently shows no signs of acute intoxication or withdrawal and is willing to consider help.

Which disposition best matches the assessment findings?

A) Facilitate urgent same-day substance-use and medical evaluation while addressing immediate overdose risk and available emergency supports

B) Begin routine weekly counseling and postpone level-of-care decisions until a personality inventory is completed

C) Schedule a reassessment in one month because the absence of current withdrawal indicates low risk

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Question 154

A licensing board sends a counselor a verified notice of complaint and requests a written response plus specified records. State law authorizes the board to obtain those records for its investigation, and the counselor's liability policy requires timely notice of licensing complaints.

What should the counselor do?

A) Notify the carrier, respond truthfully on time, and transmit authorized records securely

B) Contact the complainant and offer free sessions in exchange for withdrawing the complaint

C) Ignore the request because client confidentiality prevents cooperation with any licensing investigation

Question 155

What is a principal advantage of a well-designed structured diagnostic interview?

A) It promotes consistent coverage of relevant criteria while still requiring clinical judgment

B) It guarantees a correct diagnosis regardless of the interviewer's training

C) It eliminates the need to consider culture, context, and functional impairment

Question 156

A client asks a community counselor to connect the client's consumer sleep and mood tracker to the clinic portal. The app's terms permit sale of identifiable use data to advertising partners, and the clinic has not approved the app for clinical integration.

What should the counselor do?

A) Connect the app because the client owns the data and requested the transfer

B) Ask the client to email weekly screenshots so the app's terms no longer matter

C) Explain the privacy concern and use an approved alternative or decline the integration

Question 157

Six weeks after giving birth, a client has an elevated score on a depression screener. During interview, the client is tearful and reports guilt, reduced interest, poor concentration, and difficulty functioning, but notes that some sleep disruption occurs when the newborn wakes. The client denies thoughts of self-harm.

How should the counselor use the screening result?

A) Diagnose a depressive disorder from the total score alone without further inquiry

B) Review individual symptoms, timing, functioning, supports, and risk, and integrate the findings with the interview before forming a conclusion

C) Disregard the score because sleep disruption after childbirth makes depression screening uninterpretable

Question 158

> Mei, an autistic adult living in a group home, has authorized coordination with residential staff. Staff describe "meltdowns" during dinner. Mei says the dining room noise is overwhelming and asks to eat in a quieter area. No one has yet tracked what happens before or after the episodes.

What should the counselor facilitate?

A) A joint review of specific observations and Mei's sensory experience, followed by an agreed support trial and tracking plan

B) Acceptance of the staff's description as the clinical explanation because they observe the episodes directly

C) Separate plans for Mei and staff so neither party's interpretation influences the other's response

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Question 159

> A building evacuation interrupts a counseling session before the counselor completes the progress note. The counselor records brief contemporaneous facts and safely finishes the full note the next morning.

How should the completed documentation represent the timing?

A) Use the original session time as the note-completion time because the content concerns that session

B) Reconstruct exact quotations from memory so the delayed note appears as detailed as a routine entry

C) Identify the entry as completed later, use the actual completion time, and distinguish known facts from any limited recollection

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Question 160

> A client offers a counselor a handmade item of modest monetary value at the end of a significant cultural celebration. The agency permits small gifts when the counselor evaluates meaning, potential influence, cultural context, and clinical impact.

What should the counselor do?

A) Reject every gift automatically because acceptance always creates an unethical multiple relationship

B) Evaluate the gift under agency policy, discuss relevant meaning and boundaries, and document the decision

C) Accept the gift without discussion because low monetary value eliminates boundary concerns

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Question 161

> A counselor strongly values parenthood and notices repeatedly asking a client who plans to remain childfree whether the client might regret the decision. The client has not expressed uncertainty and wants to focus on career stress.

What should the counselor do?

A) Continue exploring possible regret because major life choices should be challenged in counseling

B) Examine the value conflict in supervision, stop steering the topic, and return focus to the client's stated goals

C) Refer the client immediately because counselors should never work with clients whose life choices differ from their own

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Question 162

> An employer asks a licensed counselor to administer and independently interpret a specialized personality test. The state's scope rule and the test publisher restrict independent use of this instrument to psychologists, and no qualified psychologist is available at the site.

What should the counselor do?

A) Administer the test because the employer assumes responsibility for scope decisions

B) Use the test if the client signs a waiver acknowledging the counselor's license type

C) Decline the restricted task and arrange an appropriate qualified provider or alternative assessment

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Question 163

A referral program temporarily pauses new intakes. Which directory update is most useful?

A) Delete the program permanently so no counselor can refer to a service that is currently unavailable

B) Record the verification date, current pause, expected recheck point, and any available alternatives

C) Leave the program marked active because temporary capacity changes are too frequent to track

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Question 164

> A transgender client worries about being harassed when using public transportation after several prior incidents. A thought record labels the prediction “Someone may target me” as irrational, and the client says the exercise feels dismissive.

How should the counselor modify the intervention?

A) Continue challenging the prediction until the client can identify evidence that public transit is generally safe.

B) Avoid discussing the fear because addressing discrimination may strengthen the client’s anxiety.

C) Distinguish realistic safety concerns from anxiety-driven predictions and develop coping and safety choices with the client.

Question 165

A practice sells a ten-session package and places advance payments into its operating account. After two sessions, a client moves away and requests the unused balance. State consumer rules require advance fees to be kept separately until earned and require prompt refund of unearned funds.

How should the practice respond?

A) Keep the balance because discounted packages become nonrefundable when the first session occurs

B) Convert the balance into a donation to the practice's reduced-fee fund

C) Refund the unearned amount and correct the handling of future advance payments

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Question 166

Kiara, a new parent, receives counseling and support from a home-visiting nurse, a lactation consultant, and a food-assistance program. She has authorized the providers to coordinate. Kiara reports repeating the same history to each provider and missing visits because appointments overlap.

What would be the most useful coordination response?

A) Have each provider keep an independent plan and ask Kiara to resolve scheduling conflicts as they occur

B) Establish client-approved shared priorities, designate a coordination contact, and align the appointment schedule

C) Ask the home-visiting nurse to control all service planning because that provider sees Kiara at home

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Question 167

An agency sometimes records sessions for clinical supervision, but recording is optional and requires separate consent. A new client believes recording is mandatory and does not know who could view a recording or when it would be deleted.

What should the counselor do before requesting recording consent?

A) Begin recording because general consent for counseling includes all supervision methods

B) Explain that recording is optional, who may access it, how it will be protected and destroyed, and the effect of declining

C) Avoid discussing supervision because details about recording could influence the client's decision

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Question 168

> Amara prefers to receive counseling in Tigrinya, and the clinic has scheduled a qualified interpreter for her intake. Before the appointment, the interpreter asks the counselor what information would be useful for preparation. The counselor has not yet met Amara.

What is the counselor's best response?

A) Briefly explain the session purpose and likely terminology, confirm the interpreter's role, and plan to speak directly with Amara

B) Provide Amara's full referral record and ask the interpreter to summarize the relevant history before the session

C) Ask the interpreter to conduct the initial symptom screening alone and report the results to the counselor

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Question 169

> An adult leaving a county behavioral health diversion program needs ongoing counseling, stable housing, and transportation to appointments. The client consents to coordination and has chosen a community clinic but has not yet contacted the housing or transportation programs.

Which transition plan best reduces the likelihood of a service gap?

A) Give the client a printed resource list and close the case on the scheduled program end date

B) Confirm appointments and eligibility with the selected services, clarify responsibilities, and address transportation before discharge

C) Keep the client enrolled in the diversion program until housing and transportation problems are permanently resolved

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Question 170

> A client uses sarcasm while describing a painful breakup. The counselor laughs briefly, and the client becomes serious and says, “I wasn’t making a joke.”

What is the counselor’s best response?

A) “Sarcasm often gives people relief from painful emotions, which is why I responded that way.”

B) “I understand now. Please continue with what happened after the breakup.”

C) “You’re right; my laugh missed how painful this is for you. I’m sorry. What was that response like for you?”

PART B — ANSWER KEY AND EXPLANATIONS

Answers correspond to the question numbers in Part A.

Question 1 — Correct: A

Correct Answer: Use a qualified language service and keep the safety plan out of the unapproved tool

Explanation: A safety plan requires accurate communication, and the organization's policy identifies qualified language assistance as the appropriate safeguard. Removing a name would not resolve retention, approval, or translation-reliability problems with the free tool.

Question 2 — Correct: C

Correct Answer: Speak directly with the client about the friend's desired role, privacy boundaries, and any specific permission before involving the friend

Explanation: The client's invitation to attend does not automatically authorize ongoing disclosure or decision-making by the friend. The counselor should center the client, define the support role, and obtain specific permission for any future involvement.

Question 3 — Correct: B

Correct Answer: Pause, acknowledge the loss of control, and present one option at a time while requesting consent before each next step.

Explanation: A trauma-informed response reduces cognitive load and restores choice after an experience involving profound loss of control. Making referrals without consent or requiring an immediate decision from a long list would reproduce pressure rather than support stabilization.

Question 4 — Correct: A

Correct Answer: Document Bao's uncertainty and coordinate qualified Vietnamese language support with the pharmacist or prescriber for the medication instructions

Explanation: Medication directions should come from the dispensing or prescribing professional in a language Bao can understand. The counselor can document the access problem and coordinate qualified language support without interpreting the dose. A general translation application may produce unsafe ambiguity in medication instructions.

Question 5 — Correct: B

Correct Answer: Practice early urge awareness and a competing response that makes pulling difficult for a brief period.

Explanation: Awareness training and a competing response directly target the cue-behavior sequence and match the client’s request for a concrete method. Public embarrassment is not a therapeutic mechanism, and symbolic analysis does not teach an alternative response to the urge.

Question 6 — Correct: C

Correct Answer: Integrate self-assessment, client-population needs, outcome information, and feedback from qualified colleagues

Explanation: Multiple sources help identify gaps that confidence or habit may miss and connect learning goals to actual practice needs. Repeating colleagues' choices or studying only familiar strengths is unlikely to produce an individualized growth plan.

Question 7 — Correct: C

Correct Answer: Activate the emergency plan, evacuate, contact responders, and account for clients and staff

Explanation: A credible bomb threat requires immediate use of the center's established safety procedure. Delaying evacuation to finish sessions or having staff search the building would expose clients and employees to unnecessary danger.

Question 8 — Correct: A

Correct Answer: Restrict the employee's access, preserve the audit log, and report the event through the required review process

Explanation: Access without a work purpose is a privacy and security event requiring containment and investigation under the stated policy. Preserving the audit evidence supports an accurate review, while waiting for proof of outside use leaves inappropriate access unaddressed.

Question 9 — Correct: B

Correct Answer: Bipolar II disorder

Explanation: The history includes a major depressive episode and a distinct hypomanic episode without any manic episode. The absence of marked impairment, psychosis, or hospitalization during the elevated period supports hypomania rather than mania under the facts provided.

Question 10 — Correct: A

Correct Answer: Follow the incident procedure immediately rather than waiting for evidence that the account was misused

Explanation: Entering credentials on the wrong site creates a credible compromise that requires prompt containment. Verified password reset, session revocation, reporting, and device review are safer than assuming normal account access proves that no intrusion occurred.

Question 11 — Correct: A

Correct Answer: Explore both commitments and help the student frame coping goals in a way that can support personal well-being and valued family relationships.

Explanation: This response makes room for the student’s relational values and individual needs without treating them as mutually exclusive. Secrecy may intensify conflict, while labeling family loyalty as a barrier imposes an individualistic standard.

Question 12 — Correct: B

Correct Answer: Current goals, interventions and responses, important client preferences, and pending follow-up needs

Explanation: These elements tell the next counselor what is active, what has been tried, how the client responded, and what still requires attention. A complete note dump can bury the current plan in unrelated history. Diagnosis and visit count alone do not support an informed continuation of care.

Question 13 — Correct: B

Correct Answer: Select evidence-focused education from qualified sources addressing current research, risks, limits, and counselor scope

Explanation: The counselor can become sufficiently informed to discuss the topic without claiming the ability to provide the service. Independent, evidence-focused education is more reliable than a local provider's marketing and more useful than avoiding an emerging topic clients are raising.

Question 14 — Correct: A

Correct Answer: Establish an accessible transit goal with the client and schedule later review of the work concern

Explanation: The client has communicated a consistent immediate priority, and supported communication is a valid means of collaborative participation. Addressing transit first respects autonomy while leaving the spouse's concern available for later review rather than allowing it to replace the client's goal.

Question 15 — Correct: C

Correct Answer: Develop an implementation intention specifying when, where, and how the client will begin with the friend’s support.

Explanation: An implementation intention turns a vague goal into a concrete cue-and-action plan and incorporates support the client has already chosen. Requiring solitary completion ignores the identified barrier, while visualizing consequences does not specify how action will start.

Question 16 — Correct: A

Correct Answer: Postpone deployment until privacy, validation, disclosure, human oversight, service limits, and crisis procedures are established

Explanation: A client-facing tool labeled as a counseling assistant can directly affect care, privacy, and crisis response. A disclaimer or an established client relationship does not replace validation, clear disclosure, human accountability, and safe escalation procedures.

Question 17 — Correct: C

Correct Answer: Provide accessible explanations, assess the client's understanding and decision-making ability, and verify whether anyone has legal authority before involving a substitute decision-maker

Explanation: A diagnosis or support need does not by itself establish incapacity. The counselor should support the client's comprehension and assess actual decision-making ability while verifying, rather than assuming, the sister's authority.

Question 18 — Correct: B

Correct Answer: Help the client create a less distressing ending and rehearse the revised dream imagery while awake.

Explanation: Imagery rehearsal involves revising the nightmare script and repeatedly practicing the new imagery during waking hours. Replaying the unchanged nightmare is not the defining technique, and symbolic interpretation is not required for this intervention.

Question 19 — Correct: B

Correct Answer: Circumstantial

Explanation: Circumstantial thought includes excessive but logically connected detail before the person eventually reaches the point. Flight of ideas involves rapid shifts among topics, whereas thought blocking is an abrupt interruption in the train of thought.

Question 20 — Correct: A

Correct Answer: Explain that counseling is collaborative, describe how the counselor and client may work together, and ask what kind of help the client expected.

Explanation: Clear orientation reduces uncertainty while inviting the client's expectations and preferences into the working relationship. Promising solutions misrepresents counseling, and immediately shifting to assessment leaves the process question unanswered.

Question 21 — Correct: B

Correct Answer: Give the required notice and ensure that compliant supervision is in place before continuing practice

Explanation: A consent order is binding even if an employer has not asked about restrictions. Informal peer contact does not satisfy an explicit supervision condition, and concealment would compound the compliance problem.

Question 22 — Correct: A

Correct Answer: Collaboratively specify the experiment, the predicted outcome, and what observations the client will record.

Explanation: A behavioral experiment is most informative when the prediction, test, and observable data are defined in advance. Reassurance asks the client to accept the counselor's belief, while using a high-stakes task introduces unnecessary risk into an initial test.

Question 23 — Correct: B

Correct Answer: Consult authoritative instrument guidance or an assessment specialist and use a supported accommodation or suitable alternative

Explanation: An apparently simple change in administration can affect standardization and interpretation. Checking authoritative guidance and consulting when needed allow the counselor to select an accessible method whose use is supported rather than improvising an accommodation.

Question 24 — Correct: A

Correct Answer: Use the agreed backup, confirm privacy, and continue or reschedule as appropriate

Explanation: The advance agreement supplies a safe continuity procedure for a routine technology failure. The counselor should use that plan and recheck privacy rather than moving clinical communication to public social media or abandoning follow-up.

Question 25 — Correct: B

Correct Answer: Review both records, note the difference, and prepare to ask Renee about her current goals and transition needs

Explanation: Both records contain relevant but time-bound information, and neither establishes Renee's present outpatient goal. Recognizing the difference prepares the counselor to clarify rather than assume. Automatically adopting or rejecting the discharge goal would place a record above Renee's current perspective.

Question 26 — Correct: B

Correct Answer: Explore scheduling and child-care barriers with the client and determine whether the agency's available supports make services workable

Explanation: Intake should identify concrete access barriers and collaboratively evaluate whether available supports make participation feasible. Prior missed appointments under different conditions do not establish unwillingness, and imposing another unworkable time would not assess suitability.

Question 27 — Correct: B

Correct Answer: Explain the limitation, refer to a qualified provider, and pursue specialized preparation only for future cases

Explanation: The required credential and the counselor's lack of specialized training make this service outside current competence. Preparation obtained prospectively may support later work, but reading standards while treating does not satisfy the present requirement.

Question 28 — Correct: A

Correct Answer: Tasha's identified concerns and goals, along with any relevant coordination need permitted by the authorization

Explanation: The authorized feedback should accurately represent Tasha's assessed concerns and client-defined goals. A referral label is not a confirmed clinical fact, particularly when the intake provides a different context. Sending the full intake would exceed the limited summary Tasha authorized.

Question 29 — Correct: B

Correct Answer: Identify and regulate the personal reaction, review it in supervision, and restore curiosity about the client's barriers

Explanation: The counselor's family experience is contributing to irritation and a lecturing stance. Recognizing and managing that countertransference creates room to understand the client's barriers instead of assuming that more information alone will change behavior.

Question 30 — Correct: C

Correct Answer: Use an accurate intern title and clearly identify the qualified supervisor and supervised status

Explanation: The reserved title cannot be borrowed from a supervisor or used for selected clients. Accurate identification prevents misrepresentation and enables clients to understand the intern's training status and supervisory structure.

Question 31 — Correct: B

Correct Answer: By translating demonstrated abilities and available supports into resources for attaining objectives

Explanation: Strengths become clinically useful when they are linked to specific goals, barriers, and action steps. Listing them separately or assuming they eliminate the need for planning does not show how they will support change.

Question 32 — Correct: B

Correct Answer: Compare the verified programs with Darius's stated criteria and let him choose among the feasible options

Explanation: Referral fit depends on the combination of service approach, transportation, scheduling, and childcare that Darius identified. Comparing verified options preserves choice while screening out services he cannot realistically use. Geographic closeness alone does not address all of the access requirements.

Question 33 — Correct: B

Correct Answer: The authorized relevant record, excluding separate notes unless specific authorization is obtained

Explanation: The facts distinguish the general medical record from separately maintained psychotherapy notes and require separate permission for the latter. Care coordination does not erase that specific authorization requirement, but it also does not prohibit an appropriately scoped clinical release.

Question 34 — Correct: A

Correct Answer: Avoid retaliation and base any care transition on clinical and ethical factors

Explanation: Conditioning treatment on withdrawal would use the counseling relationship to pressure the complainant. Any decision about continued services should rest on clinical and ethical factors, with appropriate continuity planning rather than punishment or abrupt abandonment.

Question 35 — Correct: A

Correct Answer: Explain the clinic's scope, check for urgent concerns and interest in counseling, and offer an appropriate prescribing referral

Explanation: The clinic should accurately describe its nonprescribing role while determining whether urgent needs or counseling interests are present. A referral can address medication services without misrepresenting the counselor's scope or abruptly dismissing the person.

Question 36 — Correct: A

Correct Answer: Performance in relation to defined knowledge or skill standards

Explanation: Criterion-referenced scores indicate performance against specified objectives or standards. Norm-referenced scores instead describe standing relative to a comparison group.

Question 37 — Correct: C

Correct Answer: Clarify what the diagnosis describes, correct the permanence assumption, and invite questions while setting goals together

Explanation: The client's misunderstanding is directly affecting willingness to plan treatment. Clear, collaborative review can distinguish a diagnostic course descriptor from a prediction that change is impossible and can restore informed participation.

Question 38 — Correct: B

Correct Answer: Notify the carrier promptly, preserve the record, and follow appropriate legal and claims guidance

Explanation: The written demand is a potential claim under the policy's stated notice term. Prompt reporting and record preservation protect the response process, whereas personal payment or retrospective record alteration could create additional professional and coverage problems.

Question 39 — Correct: B

Correct Answer: Explain the mandated status and reporting limits, obtain informed consent, and disclose only the required information

Explanation: A mandated client needs clear information about what will be reported, to whom, and why before counseling proceeds. The order authorizes attendance and participation reports, not unrestricted session content, so disclosure should remain within that boundary.

Question 40 — Correct: B

Correct Answer: Seek consultation to manage personal values and continue competent, nondiscriminatory counseling

Explanation: The counselor is competent to address the presenting concern, so personal discomfort alone is not a client-centered reason to refer. Consultation can help prevent value imposition and preserve equitable care without burdening the couple with the counselor's beliefs.

Question 41 — Correct: C

Correct Answer: Acknowledge the importance, clarify enough to support containment, and agree on how to return to it promptly.

Explanation: This response respects both the significance of the disclosure and the real session boundary. It avoids opening material that cannot be held adequately while ensuring the client does not experience the concern as dismissed.

Question 42 — Correct: A

Correct Answer: Replace the writing task with drawing or feeling choices followed by a brief child-led story

Explanation: The revision preserves emotional expression while matching the child's literacy level and demonstrated ways of engaging. Retaining an inaccessible task or substituting the caregiver's language would reduce the child's direct participation in treatment.

Question 43 — Correct: B

Correct Answer: Provide referral details and help the client prepare a concise summary the client may choose to share

Explanation: The client may pursue the referral without authorizing direct provider communication. Supporting client-mediated information sharing preserves autonomy and continuity, whereas contacting the specialist anyway would disregard the client's stated decision.

Question 44 — Correct: A

Correct Answer: Use multiple methods and qualified language support while considering educational opportunity, language development, culture, and measures appropriate for the student's background

Explanation: Assessment should distinguish a possible learning difficulty from language acquisition and unequal educational opportunity using appropriate, multiple sources. An inadequately normed English-only test could produce misleading conclusions, while indefinite delay would also withhold needed evaluation.

Question 45 — Correct: C

Correct Answer: Obtain a detailed developmental history and assess both autism-spectrum features and anxiety before determining the formulation

Explanation: Lifelong social-communication differences, restricted interests, sensory sensitivity, and rigidity warrant neurodevelopmental assessment. Current fear of criticism may also support anxiety, so the counselor should evaluate both rather than force an either-or conclusion prematurely.

Question 46 — Correct: B

Correct Answer: Correct the coordination error, notify the appropriate parties, and return any overpayment as required

Explanation: Duplicate payment is not earned revenue merely because both claims were processed. Correcting the ledger and resolving the overpayment with the affected payers and client supports accurate billing and prevents improper retention of funds.

Question 47 — Correct: A

Correct Answer: Restrict access as needed, report the error, and use the approved process to correct both records while preserving the audit trail

Explanation: The error affects both record accuracy and possible confidentiality, so it requires controlled correction and review. Silent deletion would erase evidence and bypass assessment, while leaving the note in the wrong chart would perpetuate the clinical risk.

Question 48 — Correct: C

Correct Answer: Compare self-reflection with supervision feedback, client-relevant outcomes, and observed performance

Explanation: Competence is better evaluated by converging information than by confidence or years in practice. External feedback, performance, and outcomes can reveal strengths or limitations that self-perception alone overlooks.

Question 49 — Correct: B

Correct Answer: Continue the safeguards and define a same-day support and treatment contact plan for strong urges or any bet

Explanation: Existing environmental safeguards reduce access to gambling, and a rapid response plan limits escalation if risk rises or a lapse occurs. Removing protections to test moderation increases exposure to a known seasonal trigger, while relying only on a partner delays the client's own response.

Question 50 — Correct: C

Correct Answer: Obtain independent education on validation, bias, privacy, human oversight, and effects on the populations served

Explanation: Evaluating a predictive system requires more than operational knowledge because its errors may distribute risks and services unfairly. Independent education on evidence, bias, privacy, and human review prepares the counselor to assess professional implications rather than relying on vendor assurances.

Question 51 — Correct: A

Correct Answer: Pause, notice the defensive reaction, invite the client's perspective, and reflect on the interaction afterward

Explanation: Brief self-regulation enables the counselor to respond to the client's experience rather than act from defensiveness. Explaining harder would bypass the feedback, while ending the session would avoid a potentially useful repair.

Question 52 — Correct: B

Correct Answer: Identify warning signs, internal coping steps, supportive contacts, professional resources, and ways to reduce access to lethal means.

Explanation: A collaborative safety plan provides a sequenced set of personalized actions and supports for managing increased risk. A no-harm promise is not a substitute for intervention, and a crisis-line number alone omits internal coping, social support, and means-safety steps.

Question 53 — Correct: C

Correct Answer: Obtain a new valid authorization before releasing the summary

Explanation: The authorization no longer permits disclosure after its stated expiration, and clinic policy requires current written permission. Altering the date or relying on the authorization's earlier validity would bypass the client's defined time limit.

Question 54 — Correct: C

Correct Answer: Document the access barrier and help Nolan request verified accessible labeling or packaging through the pharmacy and prescriber

Explanation: The problem is reliable access to medication information, not a need for the counselor to change the regimen. Accessible labels or packaging can be verified by the professionals responsible for dispensing and prescribing the medication. Relying on an uncertain visual cue could increase the chance of a medication error.

Question 55 — Correct: A

Correct Answer: Create an external cue at 7:00 p.m. and rehearse a specific sequence for pausing the game and opening the assignment.

Explanation: An external cue and rehearsed transition reduce reliance on time awareness and momentary motivation. The intervention translates the student's realistic start time into an observable sequence that can be practiced and adjusted.

Question 56 — Correct: C

Correct Answer: Consult appropriately and arrange transfer to the available clinician because the direct conflict may impair objectivity

Explanation: The counselor's close connection to an active dispute creates a substantial and specific objectivity concern. An available qualified clinician permits continuity without placing the prospective client in the position of evaluating a conflict the counselor already recognizes.

Question 57 — Correct: B

Correct Answer: Clarify the course of symptoms during intoxication, withdrawal, and abstinence before distinguishing a substance-induced condition from a primary psychotic disorder

Explanation: The timing of psychotic symptoms in relation to methamphetamine exposure and resolution during abstinence is central to the differential. Substance involvement does not justify an automatic primary diagnosis or exclusion of all psychotic disorders.

Question 58 — Correct: C

Correct Answer: Slow the pace, ask each partner to speak for themself, and explore the quieter partner's preferred language support.

Explanation: The counselor protects each partner's voice and seeks the quieter partner's own communication preference rather than assuming what support is needed. Letting one partner routinely answer for the other can conceal meaning and reinforce an unequal interaction.

Question 59 — Correct: A

Correct Answer: Explain what the note-taking is for, invite the client's reaction, and collaborate on an adjustment that supports connection.

Explanation: Transparency and collaborative adjustment respond to the client's concern without assuming that all documentation must end. Simple reassurance would fail to address the client's lived experience of disconnection.

Question 60 — Correct: B

Correct Answer: Internal consistency

Explanation: Cronbach's alpha estimates how consistently items on a multi-item scale relate to the same construct. It does not measure agreement between raters or determine how well a score predicts an external outcome.

Question 61 — Correct: B

Correct Answer: Establish a limited scheduled worry period and practice redirecting worry to that period when it appears earlier.

Explanation: Scheduled worry contains rather than suppresses the process and gives the client a structured response when worries arise. Trying to prove that every feared outcome is impossible can become another form of reassurance and is unrealistic in the face of uncertainty.

Question 62 — Correct: B

Correct Answer: Follow the custody and notice requirements and access or transfer records only through authorized processes

Explanation: Creating clinical content does not give the departing counselor personal ownership of the charts under the stated agreement and law. Authorized custody, notice, and transfer procedures protect continuity and confidentiality without copying or deleting records unilaterally.

Question 63 — Correct: A

Correct Answer: “You’re wondering whether I’m disappointed. What did you notice from me that led you to ask?”

Explanation: The response reflects the client’s concern and invites exploration of the immediate relational cue without defensively denying it. A generic statement about counselor training does not clarify what occurred between this client and counselor.

Question 64 — Correct: C

Correct Answer: Examine the rescue impulse in supervision and help the client access appropriate supports without creating personal dependency

Explanation: Shared experience may strengthen empathy, but it can also activate a rescue response that blurs professional roles. Supervision can help the counselor use the awareness constructively while connecting the client with sustainable resources rather than personal aid.

Question 65 — Correct: C

Correct Answer: The score is estimated with less precision

Explanation: The standard error of measurement reflects uncertainty around an observed score. A larger value produces a wider likely range for the person's underlying score and therefore indicates less precision.

Question 66 — Correct: A

Correct Answer: The client's needs, preferences, access, relevant evidence, and any contraindications

Explanation: Modality selection should integrate clinical fit with client preference, feasibility, evidence, and safety. A diagnostic label or counselor habit alone cannot establish that a particular modality is appropriate for an individual client.

Question 67 — Correct: C

Correct Answer: Prepare a concise handoff of effective supports, Avery's preferences, and pending needs, and arrange an introductory contact

Explanation: A focused handoff gives the receiving counselor actionable continuity information and includes Avery's preferences. An introductory contact can reduce the gap created by the move. Forwarding the entire file without selecting relevant content may obscure the supports that matter most.

Question 68 — Correct: A

Correct Answer: Explain the purpose and sequence of the intake, clarify the client's expectations, and gather sufficient biopsychosocial information before offering a formulation

Explanation: A defensible formulation and plan require enough information about history, functioning, strengths, risks, and goals. Clarifying the process respects the client's request without making a premature diagnosis or beginning an unsupported plan.

Question 69 — Correct: A

Correct Answer: Establish a shared trial of device-free meals while continuing to assess gaming, friendships, and family conflict

Explanation: The proposed trial begins with a concrete area of agreement and allows the family to gather information about the wider concerns. Adopting either the caregivers' total abstinence demand or only the adolescent's perspective would bypass collaborative family goal setting.

Question 70 — Correct: C

Correct Answer: Follow the adolescent-consent law, maintain confidentiality, and discuss permissible family involvement with the client

Explanation: The supplied law gives the adolescent both treatment consent and control of the related disclosure. Parental status alone does not override that rule, and an oral summary would still disclose confidential information.

Question 71 — Correct: C

Correct Answer: Acknowledge the distress, offer to slow the pace, and explain how the structured intake can gather necessary information without requiring a detailed narrative now

Explanation: The counselor can validate the client's experience and maintain the purpose of intake without encouraging overwhelming exposure. A detailed trauma narrative is not required to establish current needs and safety, but banning all mention of trauma would also be unresponsive.

Question 72 — Correct: B

Correct Answer: Request prompt correction and give prospective clients accurate information in the meantime

Explanation: Counselors share responsibility for addressing material public inaccuracies about their practice once discovered. Accepting referrals under a false specialty or simply blaming the insurer would leave consumers exposed to misleading information.

Question 73 — Correct: C

Correct Answer: Verify the documents and provide the records relevant to the authorized estate purpose through a secure process

Explanation: The controlling law gives a verified executor representative authority for relevant records. That authority supports a purpose-limited disclosure, not a categorical refusal or automatic release of every clinical detail.

Question 74 — Correct: B

Correct Answer: Study current adult and autism-affirming evidence, include autistic perspectives, and obtain qualified feedback on practice

Explanation: The counselor has identified outdated and incomplete preparation for the population now seeking service. Current evidence, lived perspectives, and feedback support responsive skill development more effectively than unexamined transfer of childhood methods.

Question 75 — Correct: C

Correct Answer: Verify population and purpose suitability, consult as needed, and select an appropriate alternative if support is lacking

Explanation: Assessment competence includes determining whether an instrument is supported for the examinee and the decision being made. Merely noting an unsupported use afterward does not make the resulting interpretation suitable for placement.

Question 76 — Correct: C

Correct Answer: “Your answers were clear, and I had difficulty hearing the last two. Would you like to practice them at a stronger volume?”

Explanation: Constructive feedback is specific, behaviorally observable, balanced, and connected to an immediate opportunity for practice. A global judgment about confidence is less precise and can sound evaluative rather than collaborative.

Question 77 — Correct: C

Correct Answer: Explain the limits of privacy, conduct an appropriate private portion of the interview, and combine an adolescent substance-use screen with follow-up questions

Explanation: A private, developmentally appropriate portion of the assessment can improve disclosure while preserving stated confidentiality limits. A validated adolescent screen should inform, not replace, follow-up questions and information from other relevant sources.

Question 78 — Correct: C

Correct Answer: Suspend the discriminatory routing, review affected applications, and audit and correct the system before reuse

Explanation: Automated consistency does not make a discriminatory rule acceptable. Human review, remediation for affected applicants, and correction of the system are necessary before it can again influence access to services.

Question 79 — Correct: B

Correct Answer: Name the pattern, ask the member what would support participation, and invite the group to tolerate a slower pace.

Explanation: This intervention protects the member’s opportunity to participate while inviting the member’s own preferences and making the group process visible. Requiring prepared statements shifts the burden entirely to the member, while total silence from others imposes an inflexible rule without collaboration.

Question 80 — Correct: B

Correct Answer: Record the change, the client's reason for it, the revised goal, and the related next step

Explanation: This documentation preserves the treatment history while making the current direction and client involvement clear. Replacing the old goal without context obscures how the plan evolved. A goal can change without a diagnostic change.

Question 81 — Correct: C

Correct Answer: Secure the repository, report and assess the exposure, and migrate backups to approved encrypted storage

Explanation: A working backup is not compliant when it is unencrypted and stored in an unauthorized location. The clinic must contain and assess the exposure, then restore a backup process that meets the stated security requirements.

Question 82 — Correct: A

Correct Answer: Maintain respect for the client while openly exploring the behavior, its effects, and responsibility for change.

Explanation: Unconditional positive regard concerns the client’s worth as a person, not approval of every action. The counselor can maintain acceptance while addressing harm honestly and supporting accountability.

Question 83 — Correct: B

Correct Answer: Remove the presumed goal and collaborate on the client's anxiety priorities, exploring identity only as the client wishes

Explanation: Treatment goals should reflect the client's stated needs rather than an assumed course of gender expression. The counselor can remain open to identity exploration without making a transition decision a prerequisite for anxiety care.

Question 84 — Correct: A

Correct Answer: Remind the client of the agreement and coach the client to notice anxiety without resolving the uncertainty.

Explanation: Response prevention requires interrupting reassurance seeking while helping the client remain in contact with uncertainty and anxiety. Providing reassurance would offer short-term relief but reinforce the cycle that the planned intervention is intended to change.

Question 85 — Correct: A

Correct Answer: Review current research and professional guidance, clarify training standards and scope, and seek qualified instruction before representing competence

Explanation: Mixed claims and lack of training call for a deliberate evaluation of evidence, scope, and preparation requirements. Colleague enthusiasm, equipment ownership, and tutorials do not by themselves establish competence or appropriate use.

Question 86 — Correct: C

Correct Answer: Organizing contributing and protective factors into testable hypotheses that guide goals and interventions

Explanation: A useful conceptualization explains how relevant factors may contribute to or protect against the current problem and connects those hypotheses to the plan. A chronology or diagnosis without such integration provides less individualized direction.

Question 87 — Correct: B

Correct Answer: The partners will complete a 10-minute check-in four evenings weekly and track escalation for four weeks

Explanation: This objective specifies an observable shared behavior, frequency, duration, monitoring method, and time frame that the partners consider feasible. Eliminating all disagreement is unrealistic, whereas holding check-ins only “when needed” is insufficiently measurable.

Question 88 — Correct: A

Correct Answer: The receiving service has the needed authorized information, responsibilities are clear, and the client has connected with care

Explanation: A closed-loop referral verifies that the intended connection occurred and that essential information and roles are in place. Sending paperwork or offering names documents an attempt but does not demonstrate successful linkage.

Question 89 — Correct: A

Correct Answer: Assess the images' frequency, meaning, associated rituals or avoidance, intent, insight, and any psychotic symptoms before reaching a diagnostic conclusion

Explanation: Unwanted, ego-dystonic infant-harm images with preserved insight may occur in an obsessive presentation, but risk and psychotic symptoms still require direct assessment. Neither automatically labeling the experience psychosis nor dismissing it without further inquiry is justified.

Question 90 — Correct: A

Correct Answer: “Being the person everyone depends on seems closely tied to how you measure your worth.”

Explanation: This response identifies the personal significance underlying the described responsibilities and therefore reflects meaning. The other responses accurately restate events or emotion but do not reach the client’s implied belief about worth.

Question 91 — Correct: A

Correct Answer: Maintain the planned boundary and coverage, explore the client's reaction therapeutically, and examine the counselor's guilt in supervision

Explanation: The client's response offers meaningful relational material, while the counselor's guilt may invite an exception that reinforces fears and dependency. Reliable coverage, thoughtful exploration, and supervision preserve care without abandoning an appropriate boundary.

Question 92 — Correct: A

Correct Answer: Reflect on the desire for approval, maintain the established digital boundary, and discuss the contact in counseling if clinically relevant

Explanation: The counselor's gratification could influence a response that changes the professional relationship or exposes the client connection publicly. Maintaining the boundary while addressing relevant meaning in counseling protects the relationship without using abrupt termination as a substitute for reflection.

Question 93 — Correct: A

Correct Answer: Verify the program change, update the directory, and help Mateo locate a currently verified Spanish-language option that fits his preferences

Explanation: Mateo's experience provides a concrete reason to recheck the directory and prevent the same failed referral for others. The counselor should also help restore the linkage using current, language-appropriate information. Asking Mateo to accept a service he cannot use effectively does not resolve the access barrier.

Question 94 — Correct: C

Correct Answer: Ask how the client wants the practice included and, with consent, seek culturally informed consultation before coordinating within competence

Explanation: The client is the primary source for the desired meaning and role of the traditional practice. Consent-based consultation can expand the counselor's cultural understanding and support coordination without rejecting or making assumptions about the healer's role.

Question 95 — Correct: C

Correct Answer: Explain fees, cancellation terms, and available assistance clearly enough for the client to make an informed choice

Explanation: Financial obligations and material attendance policies should be understood before the client agrees to services. The counselor should also describe the established assistance option rather than conceal costs or promise an unauthorized waiver.

Question 96 — Correct: C

Correct Answer: Maya's relevant participation, response to the intervention, and follow-up plan without identifying other members' disclosures

Explanation: Maya's individual note should show what she did, how she responded, and what she plans next. Other members' detailed disclosures are not needed to document Maya's care. A generic attendance statement would omit clinically useful information about the intervention and response.

Question 97 — Correct: B

Correct Answer: Explain confidentiality to the adult client, verify the client's own account and preferences, and obtain authorization before sharing information with the parent

Explanation: The 20-year-old is the prospective adult client and controls permission for routine disclosure. A parent's scheduling or completion of a form does not automatically grant access to counseling information.

Question 98 — Correct: A

Correct Answer: Schedule predictable check-ins, teach the direct request, reinforce its use, and track call-button frequency

Explanation: The plan addresses the apparent function of the behavior by increasing predictability and teaching a replacement request, then measures change. Removing the call button may restrict access to needed help, while inconsistent staff responses would continue unpredictability.

Question 99 — Correct: A

Correct Answer: Use the agreed backup number, confirm the client's status, and decide whether to resume or reschedule

Explanation: The documented backup arrangement exists to guide a predictable technology disruption. A disconnection alone does not establish an emergency, while simply waiting would fail to use the continuity plan both parties established.

Question 100 — Correct: A

Correct Answer: Current eligibility and availability are verified, the service fits the client's needs and preferences, and access steps are clear

Explanation: An actionable referral is current, appropriate, and specific enough for the client to use. A broad directory or an unverified agency name shifts the burden of determining basic fit and availability to the client.

Question 101 — Correct: B

Correct Answer: Explain that early reactions can include numbness, then ask what immediate support would feel most helpful.

Explanation: Numbness can be an acute response to sudden loss and does not by itself mean that the client is reacting incorrectly. Brief normalization followed by attention to the client's immediate support needs is more appropriate than forcing emotional disclosure or assigning a defensive interpretation.

Question 102 — Correct: C

Correct Answer: Invite the child to show what school has been like with the figures and let the child explain the arrangement.

Explanation: A developmentally responsive invitation uses the child's spontaneous mode of engagement while preserving the child's authority over meaning. Assigning a fixed interpretation to the figures would substitute the counselor's assumption for the child's explanation.

Question 103 — Correct: B

Correct Answer: Create distance toward the accessible exit, state a clear behavioral limit in a calm voice, and summon assistance according to protocol.

Explanation: Threatening proximity and clenched fists require the counselor to prioritize physical safety while using calm, clear communication and the stated agency procedure. Remaining seated reduces access to the exit, and escalating volume may increase danger.

Question 104 — Correct: A

Correct Answer: Elicit what protection, respect, and family connection mean to each person and identify shared hopes before negotiating differences.

Explanation: Exploring meanings and shared aims lets the family work with both autonomy and relationship values without making the counselor an arbiter. Automatically privileging either position would flatten the family's cultural and developmental complexity.

Question 105 — Correct: C

Correct Answer: Confirm an affordable intake, coordinate authorized information, and use bridge sessions and contingency resources as needed

Explanation: The foreseeable loss of coverage calls for confirmed affordable follow-up and a plan for the interval between providers. Processing the loss alone or discharging early would not establish ongoing access to care.

Question 106 — Correct: C

Correct Answer: Explore the experience's cultural meaning, context, duration, distress, impairment, and related symptoms before deciding whether it is pathological

Explanation: Bereavement experiences must be interpreted within cultural and spiritual context as well as their clinical features. The absence of impairment, fear, commands, or broader symptoms argues against reflexively assigning a psychotic diagnosis, but the experience should still be understood rather than ignored.

Question 107 — Correct: B

Correct Answer: Establish a rule-compliant employment and supervision arrangement or do not engage the associate for clinical work

Explanation: The proposed contractor and billing structure conflicts directly with the supplied associate-practice rules. Adding a supervisor's name to notes does not convert independent practice into compliant employment and supervision.

Question 108 — Correct: A

Correct Answer: Concurrent criterion validity

Explanation: Concurrent criterion validity concerns how well scores agree with an appropriate criterion measured at about the same time. Internal consistency concerns relationships among items, while test-retest reliability concerns stability across administrations.

Question 109 — Correct: C

Correct Answer: Confirm the referral, schedule permitted bridge contacts, and document coping and urgent-care options for the interim

Explanation: The six-week wait creates a foreseeable gap that can be reduced through confirmed linkage, available bridge care, and a clear interim plan. Using the remaining sessions immediately or relying on the student to manage the transition alone does not address the waiting period.

Question 110 — Correct: C

Correct Answer: To help the client experience thoughts as mental events rather than commands or literal facts.

Explanation: Cognitive defusion changes the client's relationship to thoughts by reducing automatic fusion with their literal content. It does not require proving thoughts false or tracing each one to an early event.

Question 111 — Correct: C

Correct Answer: Normalize that such reactions can occur after a frightening event, teach present-orientation skills, and explain when additional help is warranted.

Explanation: Brief normalization can reduce catastrophic interpretations while present-orientation provides a practical response to reminders. The available information does not justify declaring a disorder, and broad avoidance may maintain fear and restrict functioning.

Question 112 — Correct: C

Correct Answer: Use shorter individual sessions with paced content, accessible summaries, and checks for understanding

Explanation: The selected format responds to documented processing and memory needs as well as the client's preference. Requiring an unmodified fast-paced group or postponing all care would create unnecessary barriers rather than adapting the plan.

Question 113 — Correct: B

Correct Answer: Validate the need for relief, restate the communication boundary, and develop acceptable between-session coping and contact options.

Explanation: The counselor can respond empathically to the function of the messages while maintaining the informed boundary and building workable alternatives. Ignoring the pattern weakens the agreement, whereas abruptly blocking contact does not provide collaborative transition or coping support.

Question 114 — Correct: C

Correct Answer: Review capacity with a supervisor, adjust scheduling or workload, monitor care quality, and build a sustainable recovery plan

Explanation: Declining concentration and prolonged recovery are early functional warning signs even before a known error occurs. The response should modify contributing work conditions and monitor quality rather than depend on a brief coping exercise while preserving the same schedule.

Question 115 — Correct: B

Correct Answer: It includes clear objectives, qualified instruction, active practice, and feedback tied to current evidence

Explanation: Skill development is strengthened when learners practice observable behaviors and receive informed feedback within an evidence-based framework. Credit quantity or popularity does not show that the activity changes counseling performance.

Question 116 — Correct: B

Correct Answer: The client will clear and maintain one agreed section of the exit path within two weeks, then review the next step

Explanation: The objective is specific, safety-relevant, time-limited, and scaled to the client's current readiness while serving the meaningful long-term goal. A rapid whole-home cleanout is not presented as attainable, and postponing all action overlooks the client's willingness to begin gradually.

Question 117 — Correct: C

Correct Answer: Clarify with Nadine the roles and information she wants shared, then coordinate complementary supports within the authorization

Explanation: Nadine has asked for both forms of support and has authorized limited planning communication. Clarifying roles allows the counselor and elder to complement one another without surrendering the counseling plan or dismissing the cultural resource. The client's preferences define the purpose and boundaries of the coordination.

Question 118 — Correct: B

Correct Answer: With the client's permission, define a limited consultative role for the aunt while keeping goals client-directed

Explanation: The plan can honor the client's relational decision-making preference without transferring ownership of treatment goals. Automatically excluding family or requiring family agreement would both disregard the role the client explicitly requested.

Question 119 — Correct: C

Correct Answer: Combine developmentally appropriate interview and observation with a validated caregiver measure available in the caregiver's preferred language

Explanation: A young child's assessment should use methods suited to developmental and communication abilities while incorporating informed caregiver data. An adult self-report is inappropriate, and standardized information need not be abandoned when a suitable validated measure exists.

Question 120 — Correct: A

Correct Answer: Review the attendance and contact information and prepare neutral questions about barriers and scheduling preferences

Explanation: The record identifies a possible work-schedule barrier but does not explain the missed visits. Neutral preparation supports collaborative problem-solving based on updated information. Labeling Jordan resistant would convert an unexplained attendance pattern into an unsupported judgment.

Question 121 — Correct: C

Correct Answer: Explain the concept in plain, concrete language, allow the interpreter to convey the meaning, and check the client's understanding

Explanation: When a direct equivalent is unavailable, conveying the concept's meaning is more accurate than forcing a single-word substitution. A comprehension check helps the counselor determine whether the explanation worked. Omitting the concept would create an avoidable gap in the session.

Question 122 — Correct: A

Correct Answer: Use appropriate internal consultation and arrange assignment to another qualified counselor while limiting disclosure of personal details

Explanation: The mediator's active role in the counselor's divorce creates a direct conflict that may affect objectivity and the prospective client's privacy. Because a qualified alternative is readily available, reassignment manages the fit concern without unnecessary disclosure.

Question 123 — Correct: B

Correct Answer: Avoid clinical discussion until privacy can be established, then resume or reschedule according to policy

Explanation: The counselor should not begin sensitive work when a third party can hear and no urgency requires immediate discussion. A client's choice of location does not relieve the counselor of the duty to assess privacy and use a safer alternative.

Question 124 — Correct: B

Correct Answer: Respect the client’s decision while collaboratively developing a personalized safety plan and reviewing confidential support options.

Explanation: Client-centered safety planning supports preparation and access to help without coercing a decision the client is not ready to make. Couple counseling may be unsafe or inappropriate in a context of coercive control because open disclosure can increase risk.

Question 125 — Correct: B

Correct Answer: Obsessive-compulsive disorder

Explanation: The presentation includes intrusive contamination obsessions and time-consuming washing compulsions performed to reduce anxiety, with substantial impairment. Preserved recognition that the fears are excessive is inconsistent with the fixed belief expected in delusional disorder.

Question 126 — Correct: B

Correct Answer: Obtain affirming-practice training, examine the workflow with relevant client input, and advocate for a system correction

Explanation: The repeated misnaming reflects both a staff-learning need and a system process that is discouraging access. Training and collaborative workflow correction address the source instead of making clients repeatedly manage the burden or separating them from other clients.

Question 127 — Correct: A

Correct Answer: Explain the clinic's treatment role and limits, assess whether the person also seeks counseling, and offer referral information for a qualified independent evaluator

Explanation: The requested legal evaluation falls outside the clinic's stated role, so the mismatch should be explained before services begin. The clinic can still assess interest in treatment and provide an appropriate independent referral without promising a forensic opinion.

Question 128 — Correct: A

Correct Answer: Obtain targeted consultation from the benefits specialist while retaining responsibility for the career counseling

Explanation: The counselor has a focused factual uncertainty rather than a broad counseling-skill deficit. Consultation supplies relevant expertise while the counselor remains responsible for integrating that information into competent career counseling.

Question 129 — Correct: A

Correct Answer: Continue to protect it and disclose only when valid consent, lawful authority, or another applicable exception permits

Explanation: A client's death does not automatically make confidential counseling information public or available to every relative. The counselor should identify a valid basis and proper recipient before making any disclosure and retain or dispose of the record under applicable requirements.

Question 130 — Correct: A

Correct Answer: Use supported devices, timely security updates, approved encryption, and access controls such as multifactor authentication

Explanation: Supported, updated systems and individual access controls reduce foreseeable threats to electronic clinical information. Visible warnings are not a reliable measure of compromise, and shared credentials defeat accountability and access tracking.

Question 131 — Correct: A

Correct Answer: Conduct an immediate danger assessment and collaborate on urgent safety and support options appropriate to the level of risk

Explanation: Escalating stalking, prior strangulation, threats, firearm access, and fear of returning home indicate substantial potential danger. Diagnostic labeling or routine communication work should not displace immediate assessment and safety planning.

Question 132 — Correct: B

Correct Answer: With Ms. Patel's input, relay the functional barrier and coordinate temporary texture-appropriate meals and access to dental support

Explanation: The returned meals have a specific, potentially remediable explanation that is consistent with Ms. Patel's goal of remaining at home. Coordinating food texture and dental access addresses the barrier without mislabeling her behavior. The available facts do not establish a need for residential placement.

Question 133 — Correct: A

Correct Answer: Seek relevant training and collaborate with clients and qualified language resources to revise and test the materials

Explanation: The reported barriers call for skill development and direct evaluation of whether revised materials work for the people who use them. Leaving clients to request explanations or deleting content does not address clarity, linguistic quality, or equitable access.

Question 134 — Correct: C

Correct Answer: Confirm Luis's preferred meeting format and structure the meeting to give him response time before others contribute

Explanation: Coordination should make it possible for Luis to communicate his own preferences while still using the supports he authorized. Adjusting the meeting structure directly addresses the participation barrier he identified. Substituting his sister's voice would undermine supported decision-making rather than improve it.

Question 135 — Correct: C

Correct Answer: Coordinate relevant care and help the client distinguish episodes while retaining appropriate panic-related goals

Explanation: The new diagnosis changes the interpretation of some episodes but does not account for every panic attack described. Coordinated revision helps the client respond appropriately to both medical and anxiety-related symptoms without discarding relevant goals or ignoring new information.

Question 136 — Correct: A

Correct Answer: Scheduling appointments using privacy-protective procedures without making clinical decisions

Explanation: Scheduling is an administrative function that can be delegated with training and confidentiality safeguards. Risk triage, test interpretation, and diagnostic explanation require clinical judgment and should not be assigned to a nonlicensed administrative worker.

Question 137 — Correct: A

Correct Answer: Demonstrated skill improvement with appropriate feedback and favorable client-relevant process or outcome data

Explanation: Observable performance and relevant practice data provide stronger evidence of learning transfer than attendance, confidence, or accumulated hours. Positive data should still be interpreted in context, but they connect education to actual counseling work.

Question 138 — Correct: A

Correct Answer: Discuss foreseeable contact, agree on privacy-protective boundaries, document them, and monitor their effect

Explanation: In unavoidable community overlap, proactive discussion lets the client and counselor plan how to protect privacy and the counseling relationship. Automatic termination may unnecessarily limit access, while ignoring the issue leaves predictable boundary effects unmanaged.

Question 139 — Correct: B

Correct Answer: The result indicates that further clinical assessment is warranted

Explanation: A screening cutoff identifies elevated likelihood or concern rather than establishing a diagnosis. Diagnostic conclusions and treatment decisions require integration with interview data, impairment, context, and relevant exclusions.

Question 140 — Correct: A

Correct Answer: Confirm medication and counseling appointments, include overdose-prevention resources, and define support and reentry steps

Explanation: Confirmed follow-up, continuity of prescribed medication, overdose-prevention planning, and clear supports address the high-risk period after residential discharge. Abruptly stopping medication or providing only general numbers would weaken rather than strengthen continuity.

Question 141 — Correct: C

Correct Answer: Address the client directly, ask about communication preferences, allow sufficient response time, and involve the worker only as authorized

Explanation: Use of an augmentative device does not shift client status or decision-making to a support worker. Direct communication, adequate time, and the client's stated preferences produce a more valid and respectful intake.

Question 142 — Correct: B

Correct Answer: Review the access barrier, update backup contact and attendance plans, and reschedule while the case remains open

Explanation: The client reestablished contact before the stated closure point and identified a correctable access barrier. Revising the communication and attendance plan supports reengagement within policy, whereas automatic closure or indefinite inaction would not address continuity.

Question 143 — Correct: C

Correct Answer: Disclose the collection policy in the financial agreement and release only legally permitted information if collection becomes necessary

Explanation: Advance financial disclosure enables informed agreement and reduces surprise if a debt later goes to collection. A collection need does not justify unrestricted access to clinical records, while concealing the policy until default is not transparent business practice.

Question 144 — Correct: A

Correct Answer: Correct the field with the individualized information and verify the other carried-forward content before signing

Explanation: Automatically carried-forward text must be checked against what actually occurred in the session. Correcting the employment field and reviewing the remaining template content prevents inaccurate information from propagating. Standardization does not justify retaining a statement known to be false.

Question 145 — Correct: C

Correct Answer: Coordinate the urgent evaluation, transfer authorized information, and maintain a bridge until care is accepted

Explanation: The team has determined that the current level is inadequate, so timely coordination and a defined bridge are needed until the receiving service accepts care. Routine homework is insufficient, and immediate disengagement could create a dangerous gap.

Question 146 — Correct: B

Correct Answer: To interrupt behavior that impairs safety or productive participation while keeping the member engaged.

Explanation: Blocking sets a limit on a harmful group behavior while preserving the person’s opportunity to participate constructively. It is not a method for eliminating emotion or suppressing viewpoints that differ from the leader’s.

Question 147 — Correct: C

Correct Answer: Use the accessible evening group and, with the student's agreement, incorporate peer accountability into objectives

Explanation: This plan uses a feasible no-cost service and an existing support while preserving the student's control over peer involvement. Recommending unaffordable care or waiting for work circumstances to change would leave identified barriers unaddressed.

Question 148 — Correct: A

Correct Answer: Use supervision and team support to examine the loss of empathy, set sustainable limits, and identify realistic resource or advocacy actions

Explanation: Cynicism, blaming, guilt, and reduced empathy indicate that repeated systems frustration is affecting the counselor's work. A combined response addresses professional functioning and realistic action without emotional withdrawal or unsustainable overwork.

Question 149 — Correct: A

Correct Answer: Offer practical assistance, connection, and coping support without requiring detailed emotional disclosure.

Explanation: Early crisis support typically emphasizes current needs, social connection, and manageable coping after safety is established. Required retelling can increase distress and assumes that immediate emotional processing is necessary for everyone.

Question 150 — Correct: B

Correct Answer: Coach one partner to describe the experience while the other reflects the feeling and asks whether empathy or problem solving is wanted.

Explanation: Structured rehearsal helps the partners distinguish emotional attunement from solution giving and practice checking what response is desired. Banning all suggestions is overly rigid and would not teach the couple how to coordinate different support needs.

Question 151 — Correct: C

Correct Answer: Interrupt the comment, name its potential impact, reaffirm group expectations, and invite affected members to choose whether to respond.

Explanation: Prompt intervention protects the group climate and makes clear that demeaning speech will not be left unaddressed. Inviting rather than requiring a response avoids placing the burden of correction on members who may have been harmed.

Question 152 — Correct: C

Correct Answer: To connect members with shared themes so they can respond directly to one another.

Explanation: Linking moves communication among members by highlighting common experiences or themes. It strengthens member-to-member interaction rather than keeping the leader at the center of every exchange.

Question 153 — Correct: A

Correct Answer: Facilitate urgent same-day substance-use and medical evaluation while addressing immediate overdose risk and available emergency supports

Explanation: Daily fentanyl injection, a very recent overdose, and planned solitary use indicate high near-term overdose risk even without current withdrawal. Urgent evaluation and immediate risk-reduction supports are more appropriate than routine or delayed reassessment.

Question 154 — Correct: A

Correct Answer: Notify the carrier, respond truthfully on time, and transmit authorized records securely

Explanation: The verified board request has legal authority under the scenario, and the liability policy adds a timely notice obligation. Cooperation should be accurate, secure, and limited to what the board is authorized to obtain; bargaining with the complainant would be improper.

Question 155 — Correct: A

Correct Answer: It promotes consistent coverage of relevant criteria while still requiring clinical judgment

Explanation: Structure can improve consistency and reduce omission of important diagnostic criteria. It does not guarantee accuracy or eliminate the need to interpret culture, context, impairment, and other clinical information.

Question 156 — Correct: C

Correct Answer: Explain the privacy concern and use an approved alternative or decline the integration

Explanation: The client's request does not remove the counselor's responsibility to evaluate how a third-party app handles identifiable information. An approved alternative or no integration avoids exposing clinical data to an advertising use that the clinic has not accepted.

Question 157 — Correct: B

Correct Answer: Review individual symptoms, timing, functioning, supports, and risk, and integrate the findings with the interview before forming a conclusion

Explanation: A screening score identifies the need for fuller assessment and should be interpreted alongside symptom detail, context, functioning, and risk. Normal postpartum sleep interruption can complicate one symptom but does not make the entire result diagnostic or meaningless.

Question 158 — Correct: A

Correct Answer: A joint review of specific observations and Mei's sensory experience, followed by an agreed support trial and tracking plan

Explanation: Specific observations and Mei's account can generate a testable support plan without treating a vague label as an explanation. Tracking the agreed trial will help the team evaluate whether the quieter setting changes the pattern. Separate plans would prevent the coordinated response the situation requires.

Question 159 — Correct: C

Correct Answer: Identify the entry as completed later, use the actual completion time, and distinguish known facts from any limited recollection

Explanation: Transparent timing and careful attribution preserve the note's accuracy after an unavoidable delay. The counselor can use contemporaneous facts without pretending the entry was completed during the session. Reconstructed exact quotations would imply a level of certainty the counselor may not have.

Question 160 — Correct: B

Correct Answer: Evaluate the gift under agency policy, discuss relevant meaning and boundaries, and document the decision

Explanation: A modest gift may carry cultural and relational meaning that should be considered rather than accepted or rejected mechanically. The policy calls for a contextual assessment and documentation because low price alone does not resolve influence or boundary concerns.

Question 161 — Correct: B

Correct Answer: Examine the value conflict in supervision, stop steering the topic, and return focus to the client's stated goals

Explanation: The counselor's personal value is redirecting counseling away from the concern the client chose to address. Reflection and supervision can support value bracketing and client autonomy; difference alone does not require referral.

Question 162 — Correct: C

Correct Answer: Decline the restricted task and arrange an appropriate qualified provider or alternative assessment

Explanation: The scenario expressly places independent use outside the counselor's permitted scope, regardless of employer preference. Client waiver cannot enlarge a professional license or override the instrument's restriction.

Question 163 — Correct: B

Correct Answer: Record the verification date, current pause, expected recheck point, and any available alternatives

Explanation: A time-stamped status lets counselors avoid a currently closed pathway while preserving information that may become useful again. A recheck point and alternatives make the entry actionable. Permanent deletion would lose a potentially valuable resource because of a temporary condition.

Question 164 — Correct: C

Correct Answer: Distinguish realistic safety concerns from anxiety-driven predictions and develop coping and safety choices with the client.

Explanation: Effective cognitive work must account for credible discrimination rather than classify every feared outcome as distorted. Collaboratively separating realistic risk from anxiety amplification supports both safety and coping without invalidating the client’s experience.

Question 165 — Correct: C

Correct Answer: Refund the unearned amount and correct the handling of future advance payments

Explanation: The supplied rule makes both segregation and prompt return of unearned fees mandatory. A discount or completed first session does not authorize the practice to keep payment for services it will not provide.

Question 166 — Correct: B

Correct Answer: Establish client-approved shared priorities, designate a coordination contact, and align the appointment schedule

Explanation: A shared plan and identified coordination contact reduce repeated storytelling and prevent avoidable scheduling conflicts. Kiara's approval keeps the coordination centered on her needs. Giving one provider unilateral control would not reflect the collaborative roles of the team.

Question 167 — Correct: B

Correct Answer: Explain that recording is optional, who may access it, how it will be protected and destroyed, and the effect of declining

Explanation: Separate recording consent must be informed and voluntary under the agency's stated policy. The client needs accurate information about purpose, access, security, retention, and the consequence of declining before deciding.

Question 168 — Correct: A

Correct Answer: Briefly explain the session purpose and likely terminology, confirm the interpreter's role, and plan to speak directly with Amara

Explanation: A focused pre-session briefing helps the qualified interpreter anticipate terminology while preserving the counselor's direct relationship with Amara. Because the counselor has not assessed Amara, a full history or interpreter-led screening would be both unnecessary and role-confusing. The counselor remains responsible for conducting the intake.

Question 169 — Correct: B

Correct Answer: Confirm appointments and eligibility with the selected services, clarify responsibilities, and address transportation before discharge

Explanation: Confirmed access and clearly assigned next steps make the transition more reliable than a resource list alone. Keeping the client in a time-limited program until every social problem is permanently resolved is neither necessary nor realistic.

Question 170 — Correct: C

Correct Answer: “You’re right; my laugh missed how painful this is for you. I’m sorry. What was that response like for you?”

Explanation: The counselor directly acknowledges the misattunement, apologizes, and invites discussion of its relational impact. Explaining the laugh or quickly returning to the story would center the counselor’s intent rather than repair what the client experienced.
